

#1 · Assessment

Why is the reason for the encounter not the same as the triage chief complaint?

Answer

The written complaint is the ticket, not the agenda. What the person actually came for may take several minutes to surface. A client booked for a blood pressure recheck may be there because a partner has become frightening at home.

#2 · Assessment

What is the doorknob problem, and which interview habit prevents it?

Answer

The doorknob problem is the most serious concern appearing as you stand to leave. Prevent it by letting the person finish the opening story without interruption, then asking what else is on their list before probing any single item.

#3 · Assessment

Define pertinent history, and give the pertinent elements for chest discomfort.

Answer

Pertinent history is history that could change what you do next. For chest discomfort: onset, exertional relationship, radiation, diaphoresis, prior cardiac disease, risk factors, and recent immobility or surgery. Childhood asthma is true but not pertinent at that moment.

#4 · Assessment

What does the initial assessment establish before any detailed system examination begins?

Answer

A baseline of airway, breathing, circulation, level of consciousness and degree of distress. In a walk in clinic this can take ten seconds: how the person walks in, whether they speak in full sentences, their colour and work of breathing.

#5 · Assessment

Which presentations belong in the life threatening acuity band, and what is the time frame?

Answer

Intervention within minutes, often before the diagnosis is known. Airway compromise, severe respiratory distress, shock, major hemorrhage, anaphylaxis, status epilepticus, and suspected large vessel stroke inside the thrombolysis window.

#6 · Assessment

In a life threatening stem, why is the correct answer almost never an investigation?

Answer

Because intervention precedes diagnosis at this acuity. The right answer is an action: position the client, give oxygen, give intramuscular epinephrine, obtain vascular access, or activate emergency medical services.

#7 · Assessment

Which presentations are emergent, and how fast must you act?

Answer

Intervention within roughly an hour, diagnosing and treating in parallel rather than in sequence. Suspected sepsis, acute coronary syndrome, testicular torsion, acute angle closure glaucoma, suspected meningitis, ectopic pregnancy, and diabetic ketoacidosis.

#8 · Assessment

What qualifies as urgent, and what is the expected time to assessment?

Answer

Same day or next day attention. A first adult seizure, a hot swollen joint, a new breast mass, unexplained weight loss, or a hemoglobin that has fallen from 130 g/L to 92 g/L over six months. Delay changes the outcome.

#9 · Assessment

What belongs in the routine acuity band?

Answer

Presentations that can safely go to the next appropriate visit: stable chronic disease follow up, screening and prevention discussions, and minor self limited illness. Nothing here changes outcome if it waits for a scheduled appointment.

#10 · Assessment

Do you work up the most likely explanation or the worst plausible one?

Answer

The worst plausible explanation, until it is ruled out. A 24 year old with pleuritic chest pain and a normal chest x ray still needs pulmonary embolism considered, since the cost of missing it is death and the cost of considering it is a Wells score.

#11 · Assessment

Explain the rule that acuity is a property of the person, not of the symptom.

Answer

The same symptom sits in different bands depending on who has it. A headache is routine in a healthy 30 year old with a lifelong migraine pattern and emergent in a 68 year old with new onset pain, jaw claudication, and an elevated inflammatory response.

#12 · Assessment

Why do red flag combinations matter more than isolated red flags?

Answer

Single red flags are common and nonspecific. Fever alone is common, but fever with a petechial rash and neck stiffness is not. Back pain alone is common, but back pain with neurologic and bladder features is not.

#13 · Assessment

Which combination of back pain features is cauda equina syndrome, and what band is it?

Answer

Back pain with urinary retention, saddle anesthesia and bilateral leg weakness. It belongs in the emergent band and requires immediate imaging and surgical referral, not a trial of analgesia and reassessment.

#14 · Assessment

Is consent law in Canada federal, and what does a stem naming a province signal?

Answer

Consent law is provincial and territorial. Ontario uses the Health Care Consent Act and the Consent and Capacity Board, Quebec the Civil Code, British Columbia the Health Care (Consent) and Care Facility (Admission) Act, and common law governs elsewhere. A named province is telling you something.

#15 · Assessment

What four things does valid consent require everywhere in Canada?

Answer

It comes from a capable person, it relates to the specific treatment proposed, it is informed, and it is voluntary and free from coercion or misrepresentation.

#16 · Assessment

What information must be given for consent to count as informed?

Answer

The nature of the treatment, the expected benefits, the material risks and side effects, the alternative courses of action, and the likely consequences of declining the treatment.

#17 · Assessment

What does it mean that capacity is decision specific and time specific?

Answer

A person may lack capacity to manage finances while retaining capacity to consent to a wound dressing. A person delirious at 03:00 may be capable by 11:00 the next morning, so reassess rather than carrying an old finding forward.

#18 · Assessment

Which factors do NOT by themselves reduce a person's capacity to consent?

Answer

A diagnosis of dementia, schizophrenia or intellectual disability, and disagreement with your recommendation. A refusal you consider unwise is not evidence of incapacity. Assess capacity rather than assuming it in either direction.

#19 · Assessment

State the functional test for decision making capacity.

Answer

Whether the person can understand the information relevant to the decision, and appreciate the reasonably foreseeable consequences of deciding or of not deciding. Both limbs are required, and both are tested for the specific decision at hand.

#20 · Assessment

Give the typical ranked hierarchy of substitute decision makers.

Answer

Court appointed guardian, then attorney for personal care under a power of attorney, then tribunal appointed representative, then spouse or partner, then child or parent, then sibling, then other relative, with a public guardian as the final option.

#21 · Assessment

What standard does a substitute decision maker apply, and in what order?

Answer

The person's previously expressed capable wishes come first. Where no applicable wish exists, the substitute applies best interests, considering the person's values and beliefs, whether treatment is likely to improve or prevent deterioration, and whether expected benefit outweighs risk.

#22 · Assessment

When may treatment proceed without consent under the emergency exception?

Answer

When the person is incapable, delay would prolong suffering or risk serious bodily harm, and no substitute decision maker is available. Consent is then sought as soon as it becomes possible.

#23 · Assessment

How do Canadian jurisdictions differ on consent by minors?

Answer

Most apply the mature minor doctrine, where a young person capable of understanding the nature and consequences may consent, with no fixed age. Quebec sets 14, with reporting to parents in defined circumstances. New Brunswick sets 16 with a route for younger capable minors.

#24 · Assessment

What did A.C. v. Manitoba establish about mature minors?

Answer

The Supreme Court of Canada confirmed that a mature minor's views carry weight in proportion to demonstrated maturity. On the exam this means assessing this young person's understanding rather than looking up their birthdate.

#25 · Assessment

How does consent to share personal health information differ from consent to treatment?

Answer

They are separate. Provincial privacy statutes such as Ontario's Personal Health Information Protection Act and Alberta's Health Information Act generally allow implied consent inside the circle of care and require express consent outside it. Telling a helpful family member who is not the substitute decision maker is a breach.

#26 · Assessment

What is critical inquiry, and what does the biopsychosocial layer add?

Answer

Critical inquiry is the deliberate integration of everything shaping health, not only the presenting system. Insomnia, palpitations and weight loss may be thyrotoxicosis, and may also be three precarious shifts plus caring for a dying parent. Both deserve testing, and a TSH settles one branch cheaply.

#27 · Assessment

Why do social determinants of health change management, not just context?

Answer

Income, housing, food security, education, employment, early childhood experience, social inclusion and access to services predict outcomes more powerfully than most clinical variables. There is no point prescribing a medication a person cannot afford, or advising refrigerated insulin storage to someone living unsheltered.

#28 · Assessment

Distinguish cultural competence, cultural humility and cultural safety.

Answer

Competence implies a culture can be learned as knowledge. Humility is lifelong self reflection. Safety is the outcome, and it is judged by the person receiving care, not by the provider who delivered it.

#29 · Assessment

What do Truth and Reconciliation Commission Calls to Action 18 through 24 address?

Answer

Health. Call 18 recognizes health harms from residential schools and government policy. Calls 19 to 22 address measuring and closing outcome gaps, sustaining Aboriginal healing centres, and recognizing Indigenous healing practices. Calls 23 and 24 address the Indigenous health workforce and mandatory training in Indigenous health, rights and anti racism.

#30 · Assessment

What does practical cultural safety look like in an assessment encounter?

Answer

Asking permission before touching, explaining each step, not assuming who should be in the room, offering to include a family member or Elder, and never recording an assumption about substance use or non adherence that the record cannot support. Joyce's Principle made these stakes concrete.

#31 · Assessment

How is an encounter structured under trauma informed care, and is a disclosed history needed?

Answer

Assume any person may have a trauma history and structure the encounter around predictability, choice and control. Say what you are about to do and why, state that the person may stop at any point, offer a chaperone, and consider deferring a sensitive examination. No disclosure is required.

#32 · Assessment

What is an organ inventory, and why does it beat reading the health card sex marker?

Answer

It is a direct record of which organs the person actually has, taken rather than assumed. A transgender man may still need cervical screening, and a transgender woman on long term estrogen may still need prostate consideration. Also ask and record the name and pronouns used.

#33 · Assessment

What does a complete pharmacological history cover?

Answer

Prescription medications, over the counter products, cannabis, herbal and natural health products, borrowed or shared medications, adherence in practice rather than in theory, prior adverse drug reactions with what actually happened, and who prescribes what, since several prescribers raise interaction and duplication risk.

#34 · Assessment

Why does mislabelling a side effect as an allergy matter?

Answer

Many so called allergies are known side effects. Recording one as an allergy narrows future treatment options unnecessarily. Ask what actually happened during the reaction rather than accepting the label on the chart.

#35 · Assessment

Which advance care planning questions belong in routine assessment, not only palliative care?

Answer

Whether the person has thought about who would speak for them, whether that person knows their wishes, and whether anything is written down. In serious illness, explore what a good day looks like, which abilities matter most, and what trade offs they would accept.

#36 · Assessment

What drives the choice between a comprehensive and a focused assessment?

Answer

Acuity, how much you already know about the person, and how much diagnostic uncertainty remains. Comprehensive is not automatically safer: a thirty minute head to toe examination on a client in respiratory distress is a harm, and so is a focused assessment that never asks about medications.

#37 · Assessment

What do screening tools produce, and what does choosing a sensitive tool commit you to?

Answer

Probabilities, not diagnoses. Tools must be valid and reliable in the population in front of you. A sensitive tool is chosen to rule out, and it will produce false positives that need follow up, so plan for that before you order it.

#38 · Assessment

Name three low value tests Canadian stewardship guidance argues against.

Answer

Routine annual bloodwork in an asymptomatic adult, imaging for uncomplicated low back pain without red flags, and general population vitamin D testing. Choosing Wisely Canada and the Canadian Task Force on Preventive Health Care both push against reflex panels. Overtesting harms through incidentalomas, cascades, cost and anxiety.

#39 · Assessment

Give the SI reference ranges for adult creatinine, fasting glucose and hemoglobin.

Answer

Creatinine roughly 60 to 110 $\mu\text{mol/L}$ for men and 45 to 90 $\mu\text{mol/L}$ for women. Fasting glucose in mmol/L , with impaired fasting glucose 6.1 to 6.9 mmol/L and diabetes at 7.0 mmol/L or above on a confirmed sample. Hemoglobin near 135 to 175 g/L for men and 120 to 160 g/L for women.

#40 · Assessment

What are the folic acid and medication review priorities in preconception assessment?

Answer

Folic acid usually 0.4 mg daily starting two to three months before conception and through the first trimester, with higher doses after a neural tube defect affected pregnancy or with diabetes, obesity, malabsorption or certain anticonvulsants. Review teratogens: ACE inhibitors, angiotensin receptor blockers, isotretinoin, methotrexate, valproate.

#41 · Assessment

Which infant temperature is emergent, and which Canadian tool structures well baby visits?

Answer

Any fever of 38.0 degrees Celsius or higher under three months of age is emergent and requires a full septic workup rather than reassurance. The Rourke Baby Record structures early visits, with growth plotted on World Health Organization charts adapted for Canada.

#42 · Assessment

In young child assessment, which developmental finding is always abnormal?

Answer

Loss of previously acquired skills. Developmental surveillance happens at every visit, with standardized screening where concern arises, covering speech and language, gross and fine motor skills, social interaction and behaviour. Also screen for iron deficiency risk from excessive cow's milk, dental caries, and adverse childhood experiences.

#43 · Assessment

What does HEEADSSS stand for, and how should the interview be sequenced?

Answer

Home, Education and employment, Eating, Activities, Drugs, Sexuality, Suicide and depression, Safety. Offer time alone without a caregiver at every visit, explain confidentiality and its limits first, and move from lower risk domains to higher risk ones so trust builds before the hardest questions.

#44 · Assessment

How does assessment change for the older adult?

Answer

Move from single disease thinking to function. Screen frailty with a validated tool such as the Clinical Frailty Scale, and assess falls history, gait and balance, cognition, continence, hearing and vision, nutrition, mood, social isolation and driving. Review medications with explicit deprescribing intent.

#45 · Assessment

Why might screening colonoscopy be the wrong offer in advanced age?

Answer

It offers no benefit to a person whose life expectancy is shorter than the ten years the benefit takes to appear, while carrying immediate procedural risk. Weigh burden against benefit for every test and medication, prioritize comfort, function and stated goals, and confirm the substitute decision maker.

#46 · Assessment

Which three questions, in order, decide how wide to cast the assessment net?

Answer

What could kill or disable this person today, what is most likely, and what finding would change my plan right now. A maneuver that cannot change management is a maneuver you can defer.

#47 · Assessment

Which constitutional symptoms stop an assessment from being routine?

Answer

Unintentional weight loss, drenching night sweats, fever without a source, and progressive fatigue. They are the shared prefix of malignancy, tuberculosis, HIV, endocarditis and inflammatory disease, and two or more together change the assessment.

#48 · Assessment

Why document pertinent negatives and not just positives?

Answer

Recording that a client with headache has no fever, no neck stiffness and no focal deficit proves the dangerous diagnoses were considered. On a licensure exam and in a chart audit, a missing pertinent negative reads as an assessment never done.

#49 · Assessment

List the Centor criteria and the Mclsaac age modification.

Answer

One point each for fever above 38 degrees Celsius, absence of cough, tender anterior cervical adenopathy, and tonsillar exudate or swelling. Mclsaac adds one point for age 3 to 14 and subtracts one for age 45 and older.

#50 · Assessment

What action does each Centor and Mclsaac score band support in Canadian primary care?

Answer

A score of 0 or 1 means no swab and no antibiotic. A score of 2 or 3 supports a swab with treatment guided by the result. A score of 4 or 5 still favours swabbing over empiric treatment, matching Choosing Wisely Canada's advice against antibiotics without confirmed group A streptococcus.

#51 · Assessment

Which sore throat findings outrank the Centor score entirely?

Answer

Trismus, a muffled voice, unilateral swelling with uvular deviation and drooling suggest peritonsillar abscess or deep neck space infection. Stridor, tripod positioning and severe pain with a benign looking oropharynx suggest epiglottitis. None of these are swab situations, they are transfer situations.

#52 · Assessment

Which four findings separate a benign red eye from a sight threatening one?

Answer

Visual acuity, pain, pupil, and the pattern of redness. Measure acuity first every time, since it is the vital sign of the eye. Ciliary flush, a fixed mid dilated pupil, a hard globe, photophobia and reduced acuity point to angle closure glaucoma, anterior uveitis or keratitis.

#53 · Assessment

Which hearing loss pattern is normal ageing and which patterns are not?

Answer

Presbycusis, a symmetric high frequency sensorineural loss with disproportionate trouble in a noisy room, is normal age related change. Asymmetric or unilateral sensorineural loss warrants investigation for vestibular schwannoma. Sudden sensorineural loss over hours to days is an emergency, since corticosteroid treatment is time sensitive.

#54 · Assessment

Which lymph node features mean malignancy until proven otherwise?

Answer

A firm, fixed, non tender node larger than 1 cm persisting beyond four to six weeks in an adult, especially a supraclavicular node. Neck examination always includes the thyroid and all node chains.

#55 · Assessment

Give the ABCDE melanoma criteria, and say which one carries the most weight.

Answer

Asymmetry, border irregularity, colour variegation, diameter above 6 mm, and evolution. Evolution is the most powerful, since a changing lesion beats a static odd looking one. The ugly duckling sign, a lesion that does not match the person's other moles, catches melanomas ABCDE misses.

#56 · Assessment

Which three skin presentations signal a systemic emergency?

Answer

Non blanching petechiae with fever suggest meningococemia. Rapidly spreading erythema with pain out of proportion, crepitus and systemic toxicity suggests necrotizing fasciitis, where marking the border and reassessing within the hour is part of the assessment. Target lesions with mucosal involvement suggest Stevens Johnson syndrome.

#57 · Assessment

What does respiratory inspection tell you before the stethoscope, and how long do you count the rate?

Answer

Respiratory rate, work of breathing, accessory muscle use, ability to speak in full sentences and tripod positioning all indicate severity before auscultation. Count the rate for a full 30 seconds.

#58 · Assessment

Match wheeze, coarse crackles, fine crackles, consolidation and tension pneumothorax to their findings.

Answer

Wheeze is a narrowed airway. Coarse crackles clearing with cough are secretions. Fine late inspiratory crackles that do not clear suggest interstitial disease or pulmonary edema. Bronchial breath sounds peripherally with increased fremitus and dullness mean consolidation. Absent sounds with hyperresonance and tracheal deviation away mean tension pneumothorax.

#59 · Assessment

Why is a silent chest in a client with known asthma so dangerous?

Answer

It is the most misread respiratory finding. Reduced wheeze with a rising carbon dioxide level and a falling level of consciousness means the client is exhausted and moving too little air to make noise, not that the asthma is improving.

#60 · Assessment

Name the components of the Wells criteria for pulmonary embolism.

Answer

Clinical signs of deep vein thrombosis, pulmonary embolism being the most likely diagnosis, heart rate above 100, immobilization or surgery within four weeks, previous venous thromboembolism, hemoptysis, and malignancy.

#61 · Assessment

When is a D dimer the wrong test in suspected pulmonary embolism?

Answer

In a high probability client, because a negative result cannot lower the probability enough to stop the workup. A low probability score with negative PERC criteria supports no further testing, and Choosing Wisely Canada advises against imaging or D dimer in low probability clients who meet PERC.

#62 · Assessment

In which situations can pulse oximetry read normal and still mislead you?

Answer

Early pulmonary embolism, carbon monoxide poisoning, and anemia. It is also less reliable on darker skin pigmentation. Treat a normal saturation as one data point, never as reassurance that overrides the story.

#63 · Assessment

Describe correct blood pressure technique and the meaning of an interarm difference.

Answer

Correctly sized cuff, client seated with back supported and feet flat, arm at heart level, no talking. Measure both arms at the first visit, since a difference above 15 mmHg suggests subclavian stenosis or aortic dissection. Canadian practice favours automated office and out of office measurement.

#64 · Assessment

How is jugular venous pressure measured, and what does isolated peripheral edema suggest?

Answer

Position at 30 to 45 degrees, identify the internal jugular pulsation rather than the carotid, and measure vertical height above the sternal angle. Elevated JVP with edema, an S3 and crackles supports heart failure. Edema alone with normal JVP more often means venous insufficiency, calcium channel blocker effect, or hypoalbuminemia.

#65 · Assessment

How do maneuvers separate aortic stenosis from hypertrophic cardiomyopathy?

Answer

A systolic murmur radiating to the carotids with a delayed, diminished carotid upstroke suggests aortic stenosis. A murmur louder with Valsalva and quieter with squatting suggests hypertrophic cardiomyopathy, because reduced preload worsens the obstruction.

#66 · Assessment

Which murmurs need an echocardiogram, and which do not?

Answer

Any diastolic murmur is pathologic and needs an echocardiogram. Choosing Wisely Canada advises against echocardiography for an asymptomatic client with an innocent sounding systolic murmur.

#67 · Assessment

Which chest pain features raise risk, and who presents atypically?

Answer

Exertional onset, radiation to both arms or the jaw, diaphoresis and vomiting. Reproducible chest wall tenderness lowers probability but does not eliminate it. Women, older adults and people with diabetes skew toward dyspnea, fatigue and epigastric discomfort, so an atypical story in a high risk person deserves an ECG.

#68 · Assessment

Give the abdominal examination sequence and the classic quadrant patterns.

Answer

Inspect, auscultate, percuss, then palpate, beginning furthest from the reported pain and watching the face. Right upper quadrant pain with a positive Murphy sign suggests cholecystitis, periumbilical migration to the right lower quadrant with anorexia suggests appendicitis, and left lower quadrant pain with fever in an older adult suggests diverticulitis.

#69 · Assessment

Which abdominal triad is a ruptured aortic aneurysm, and when does imaging happen?

Answer

A pulsatile expansile mass with back pain and hypotension. Imaging happens only if the client is stable enough, since transfer and resuscitation come first.

#70 · Assessment

List the upper gastrointestinal alarm features that mean endoscopy rather than an acid suppression trial.

Answer

Dysphagia, odynophagia, unintentional weight loss, persistent vomiting, bleeding, iron deficiency anemia, a palpable mass, and new onset symptoms after age 50.

#71 · Assessment

Which bowel changes are normal ageing and which are never attributed to age?

Answer

Slower colonic transit and reduced rectal sensation make constipation more common with age. New onset constipation with a change in stool calibre, rectal bleeding, or iron deficiency anemia is never attributed to age, and iron deficiency anemia in a man or postmenopausal woman means a colon cancer workup.

#72 · Assessment

How do you separate cystitis from pyelonephritis, and what is the rule on asymptomatic bacteriuria?

Answer

Dysuria, frequency and urgency without fever suggest cystitis, while fever, rigors, flank pain and costovertebral angle tenderness suggest pyelonephritis. Choosing Wisely Canada is explicit that asymptomatic bacteriuria is not screened for or treated outside pregnancy and certain urologic procedures, so a positive culture does not explain a fall.

#73 · Assessment

What are the five P areas of a sexual health history?

Answer

Partners, practices, protection, past sexually transmitted infection, and pregnancy intention. Ask about gender of partners rather than assuming, and use the client's own words for anatomy.

#74 · Assessment

Which testicular findings mean torsion, and what must not delay urology?

Answer

Sudden severe testicular pain with a high riding testis and an absent cremasteric reflex. This is a surgical emergency, and ultrasound must not delay urology consultation.

#75 · Assessment

Name the pregnancy red flags by trimester timing.

Answer

First trimester vaginal bleeding, severe abdominal pain and shoulder tip pain suggest ectopic pregnancy. After 20 weeks, headache, visual disturbance, right upper quadrant pain and blood pressure of 140/90 mmHg or higher suggest preeclampsia, with hyperreflexia and clonus signalling severity. Reduced fetal movement after 28 weeks always warrants same day assessment.

#76 · Assessment

What is the sequence of a musculoskeletal examination, and what is checked twice?

Answer

Look, feel, move, then special tests, always including the joint above and the joint below. Neurovascular status is checked before and after any manipulation.

#77 · Assessment

State the Ottawa Ankle Rules and the Ottawa Foot Rules.

Answer

Ankle radiography for malleolar zone pain plus either bone tenderness at the posterior edge or tip of either malleolus, or inability to bear weight for four steps both immediately and in the assessment setting. Foot radiography for midfoot zone pain plus tenderness at the base of the fifth metatarsal or the navicular, or the same weight bearing criterion.

#78 · Assessment

State the Ottawa Knee Rule, and explain why a negative Ottawa rule is trusted.

Answer

Radiography for age 55 or older, isolated patellar tenderness, fibular head tenderness, inability to flex to 90 degrees, or inability to bear weight for four steps. These rules are highly sensitive and were designed to be safe rather than precise, which is why a negative rule reliably rules out fracture.

#79 · Assessment

Who does the Canadian C-Spine Rule apply to, and what are the high risk factors?

Answer

Alert, stable trauma clients. Imaging is required for any high risk factor: age 65 or older, a dangerous mechanism, or paresthesias in the extremities.

#80 · Assessment

In the Canadian C-Spine Rule, what are the low risk factors and the rotation test?

Answer

A simple rear end collision, a sitting position in the emergency department, ambulation at any time, delayed onset neck pain, or absence of midline tenderness. If one is present, ask the client to rotate the neck 45 degrees in each direction, and successful rotation means no imaging.

#81 · Assessment

Which features trigger imaging under the Canadian CT Head Rule in minor head injury?

Answer

A Glasgow Coma Scale below 15 at two hours, suspected open or depressed skull fracture, signs of basal skull fracture, two or more episodes of vomiting, age 65 or older, retrograde amnesia beyond 30 minutes, or a dangerous mechanism.

#82 · Assessment

What is the low back pain red flag hunt, and when is early imaging indicated?

Answer

Saddle anesthesia, urinary retention with overflow incontinence and bilateral leg weakness indicate cauda equina, needing immediate imaging and surgical referral. Fever with injection drug use suggests spinal epidural abscess, and night pain with weight loss and cancer history suggests metastasis. Without red flags, imaging in the first six weeks changes nothing.

#83 · Assessment

Which single neurological screen carries the highest yield, and why?

Answer

Gait, because it integrates strength, proprioception, cerebellar function and vision at once. The full structure is level of consciousness, cranial nerves, motor, sensory, reflexes, coordination and gait, answering two questions: is there a lesion, and where is it.

#84 · Assessment

What does a thunderclap headache suggest, and what determines stroke treatment eligibility?

Answer

A headache reaching maximum intensity within a minute suggests subarachnoid hemorrhage. For a sudden focal deficit, the time of last known well determines eligibility for thrombolysis and thrombectomy. Use a validated screen such as FAST, then move immediately to imaging.

#85 · Assessment

Contrast upper and lower motor neuron findings in a weak limb.

Answer

Upper motor neuron findings are spasticity, hyperreflexia and an upgoing plantar response. Lower motor neuron findings are flaccidity, atrophy, fasciculations and hyporeflexia. Sorting a weak limb into one category narrows the differential faster than any other step.

#86 · Assessment

Compare the symptom patterns of hypothyroidism and hyperthyroidism, including in older adults.

Answer

Hypothyroidism shows fatigue, cold intolerance, constipation, weight gain, dry skin and a delayed ankle reflex relaxation phase. Hyperthyroidism shows weight loss despite good appetite, heat intolerance, tremor, and atrial fibrillation in older adults, where apathetic presentations are common. Choosing Wisely Canada advises against routine screening in asymptomatic adults.

#87 · Assessment

Give the SI diagnostic thresholds for diabetes and the core annual assessment.

Answer

A fasting plasma glucose of 7.0 mmol/L or higher, a two hour value of 11.1 mmol/L or higher, or an A1C of 6.5 percent or higher. The annual core is a foot examination with 10 g monofilament testing, blood pressure, retinal screening, and an albumin to creatinine ratio.

#88 · Assessment

Where does hematologic assessment start, and what does a low ferritin demand?

Answer

Pallor of the conjunctivae and palmar creases, then a search for the cause. Interpret hemoglobin in g/L, ferritin in mcg/L, creatinine in umol/L and electrolytes in mmol/L. A low ferritin demands a source, not just a supplement. Easy bruising and petechiae suggest a platelet problem, while joint and muscle bleeding suggests factor deficiency.

#89 · Assessment

What can genuinely be assessed by video?

Answer

History, mental status, respiratory effort, speech, level of distress, gait across a room, skin lesions in good lighting, and observed self measurement of blood pressure, weight, temperature and glucose. Client guided maneuvers such as pointing to the pain site or pressing on the abdomen while you watch the face add real value.

#90 · Assessment

What cannot be done virtually, and what must be documented at the end of a virtual visit?

Answer

Auscultation, palpation for masses and organomegaly, jugular venous pressure, fundoscopy, otoscopy and internal examination. The Ottawa rules and the Canadian C-Spine Rule need palpation and supervised movement, so video cannot apply them to defer imaging. Document what could not be assessed, the safety netting given, and the threshold for in person care.

#91 · Assessment

Distinguish the mental health history from the mental status examination, and say where the clinical value sits.

Answer

The history is what the client tells you across time. The mental status examination is what you observe and elicit right now, repeatable over time. Value comes from the mismatch: a client reporting feeling fine while showing psychomotor retardation and restricted affect is telling you two things, and the observed data usually deserves more weight.

#92 · Assessment

List the domains of the mental status examination.

Answer

Appearance and behaviour, motor activity, speech, mood as the client names it, affect as you observe it, thought process and content, perception, cognition, insight, and judgment.

#93 · Assessment

What should new onset psychosis or a first manic presentation after age 40 push you toward first?

Answer

Non psychiatric causes before a primary psychiatric diagnosis: delirium, medication effects, substance intoxication or withdrawal, endocrine disease, infection, and intracranial causes. The same applies to new confusion at any age.

#94 · Assessment

Which bedside feature best discriminates delirium from a primary psychiatric disorder?

Answer

Fluctuating attention. Delirium waxes and wanes and impairs attention early, while a primary psychiatric disorder usually preserves orientation and attention.

#95 · Assessment

What is the PHQ-9 for, what is its threshold, and what is its failure mode?

Answer

Nine items mapped to the DSM criteria for major depression, scored 0 to 27, with 10 or higher the usual threshold, best used for severity and treatment response after diagnosis by interview. Somatic items inflate scores in cancer, chronic pain, sleep apnea, anemia and pregnancy, and a zero on item 9 does not clear suicide risk.

#96 · Assessment

What does the GAD-7 measure well, and where does it fail?

Answer

Seven items scored 0 to 21, threshold of 10 for probable generalized anxiety disorder, and a good severity tracker. It is much weaker for panic disorder, social anxiety disorder, obsessive compulsive disorder and post traumatic stress disorder, so a low score in an obviously anxious client should not close the assessment.

#97 · Assessment

Why does the EPDS beat the PHQ-9 in the perinatal period, and what is the usual cut point?

Answer

Ten items scored 0 to 30, used antenatally and postpartum and validated in partners as well as birthing parents. It omits somatic symptoms, which matters in a period where fatigue and broken sleep are near universal. A cut point of 13 or higher is common, lower when higher sensitivity is wanted, and item 10 addresses self harm.

#98 · Assessment

Which postpartum emergency does the EPDS not screen for, and how does it present?

Answer

Postpartum psychosis, a psychiatric emergency of confusion, rapid mood swings and delusions, often within the first two weeks. A reassuring EPDS score gives no protection against it.

#99 · Assessment

What does the MDQ screen for, when is it used, and what is its failure mode?

Answer

A lifetime history of hypomania or mania. A positive screen needs seven or more symptom items clustering in the same period plus at least moderate functional problems. Use it before starting an antidepressant in recurrent or early onset depression or poor antidepressant response. It false positives often in borderline personality disorder and active substance use.

#100 · Assessment

What are the AUDIT-C thresholds, and what exactly does a positive result mean?

Answer

Three consumption items scored 0 to 12, with common positive thresholds of 4 or higher for men and 3 or higher for women. It screens for hazardous drinking, not alcohol use disorder, so a positive result opens an assessment rather than closing one.

#101 · Assessment

What does CRAFFT stand for, who is it for, and what invalidates it?

Answer

Car, Relax, Alone, Forget, Family or Friends, and Trouble, a youth screen for roughly ages 12 to 21. Two or more affirmative answers signals a fuller assessment, and the Car item alone is a safety concern regardless of total. It depends on a confidential interview and loses validity when a parent stays in the room.

#102 · Assessment

What is the Columbia Suicide Severity Rating Scale good for, and what can it not do?

Answer

It grades ideation from a passive wish to be dead through active ideation with plan and intent, and covers actual, interrupted and aborted attempts and preparatory acts. Its value is shared language across a team and a record of what you asked. It is not predictive, and a low score is not a clearance.

#103 · Assessment

Compare the MoCA and the MMSE, including what neither one can do.

Answer

Both are 30 point screens. The MoCA is more sensitive to mild cognitive impairment and executive dysfunction, includes trail making, clock drawing and delayed recall, adds one point for twelve or fewer years of education, and needs training. The MMSE misses executive impairment. Neither distinguishes delirium from dementia or determines capacity.

#104 · Assessment

Why is the Geriatric Depression Scale suited to older adults, and when does it lose validity?

Answer

The 15 item short form uses yes or no answers and avoids somatic items, which suits older adults with multimorbidity, and a score of 5 or higher warrants further assessment. It loses validity in moderate to severe dementia, where collateral history matters more.

#105 · Assessment

What is the Canadian position on blanket depression screening, and what must pair with every positive screen?

Answer

The Canadian Task Force on Preventive Health Care does not recommend routinely giving depression questionnaires to all asymptomatic adults, so use tools for case finding and severity tracking. Pair every positive screen with a medical differential: hypothyroidism, anemia and B12 deficiency all mimic depression, reported as TSH in mIU/L, hemoglobin in g/L and B12 in pmol/L.

#106 · Assessment

Why abandon low, medium and high suicide risk stratification, and what replaces it?

Answer

Stratification of that kind performs poorly at the individual level, and most people who die by suicide were rated low risk at their last contact. Replace it with a formulation: these drivers, these protective factors, this access to means, and this plan for the next few days.

#107 · Assessment

Name the static suicide risk factors, and identify the strongest single predictor.

Answer

Previous attempt, which is the strongest single predictor, plus male sex for death by suicide, family history of suicide, childhood abuse, chronic mental illness, and chronic pain or disabling physical illness. Static factors set a baseline and do not change with your intervention.

#108 · Assessment

Name the dynamic suicide risk factors, and say why they matter clinically.

Answer

Current ideation with plan and intent, hopelessness, agitation and severe insomnia, acute intoxication, recent discharge from psychiatric care, acute loss such as relationship breakdown, job loss or eviction, and new access to lethal means. They change, and they are where your intervention lives.

#109 · Assessment

How should protective factors be weighted in a suicide risk formulation?

Answer

They are not a subtraction from risk. Connection to family or community, responsibility for dependants, engagement in care and reasons for living all matter, but they collapse under acute intoxication or agitation. Never let a protective factor override an active plan.

#110 · Assessment

Which suicide prevention intervention has the strongest evidence, and what does it look like in practice?

Answer

Means restriction. Ask what the person can reach: firearms in the home, stored medications including acetaminophen and tricyclics, ropes, vehicles. Negotiate concrete steps such as transferring firearms to another person, weekly blister pack dispensing, or having family hold supplies. Firearms concerns can trigger reporting to a chief firearms officer under the Firearms Act.

#111 · Assessment

What are the defined components of a safety plan?

Answer

A written, collaborative, client owned document containing personal warning signs, internal coping strategies, people and settings that provide distraction, people to contact for help, crisis services with real phone numbers, and the agreed means restriction steps. It gives the person something to do in the moment.

#112 · Assessment

Why is a no suicide contract always the wrong exam option?

Answer

It has never been shown to reduce suicide or attempts, it shifts responsibility onto the client, it discourages honest disclosure because breaking a promise feels like failure, and it gives false reassurance. Choose safety planning plus means restriction plus a defined follow up interval.

#113 · Assessment

Which Canadian case governs duty to warn, and what three considerations does it turn on?

Answer

Smith v. Jones, a 1999 Supreme Court of Canada decision. It turns on whether there is a clear risk to an identifiable person or group, whether the threatened harm is serious bodily harm or death, and whether the danger is imminent.

#114 · Assessment

Once the duty to warn threshold is met, what and to whom do you disclose?

Answer

The minimum information needed to prevent the harm, to those who can prevent it, usually police and the identified person, with the reasoning and threshold applied documented. A vague statement of anger with no identifiable target does not meet the test, and disclosing anyway is a privacy breach.

#115 · Assessment

How do provincial and territorial mental health acts differ for involuntary assessment?

Answer

They differ meaningfully in criteria, duration and review. Ontario uses a Form 1 application allowing up to 72 hours of assessment, British Columbia uses medical certificates, and Alberta uses admission certificates. Some jurisdictions, British Columbia among them, authorize nurse practitioners to complete the certificate, while others, Ontario among them, restrict the application to physicians.

#116 · Assessment

What three questions does substance use assessment answer, and what must be asked about route?

Answer

What is used, how much and how often, and what harm results. Cover alcohol, cannabis, prescribed and non prescribed opioids, stimulants, benzodiazepines, nicotine and vaping, and another person's medication. Ask about route, since injection use changes infectious disease screening and overdose risk. Ask without judgment, since expected judgment drives underreporting.

#117 · Assessment

Which withdrawal syndromes can kill, and what predicts a severe course?

Answer

Alcohol and benzodiazepine withdrawal can kill, while opioid withdrawal is severely uncomfortable but rarely fatal. Prior withdrawal seizures or delirium predict recurrence, so ask about them explicitly.

#118 · Assessment

How are capacity and cognition different, and what makes capacity time specific?

Answer

Capacity is decision specific: a person may lack capacity for complex finances while keeping capacity to consent to a wound dressing. It fluctuates with delirium, pain, sedating medication, hypoglycemia and sleep deprivation, so a person found incapable during a delirium must be reassessed once it resolves.

#119 · Assessment

What is the starting presumption about capacity in Canada, and what is the general test?

Answer

Capacity is presumed. The general test, expressed in Ontario as the two part test under the Health Care Consent Act and phrased similarly elsewhere, asks whether the person can understand the information relevant to the decision and appreciate the reasonably foreseeable consequences of deciding or not deciding. A refusal you disagree with is not evidence of incapacity.

#120 · Assessment

A client scores low on the MoCA. Is that a capacity assessment, and what do you do next?

Answer

No. A low score should prompt a capacity assessment for the specific decision at hand, using a structured approach such as the Aid to Capacity Evaluation, plus steps to maximize capacity: treat the delirium, correct hearing and vision, use plain language, involve an interpreter, and reassess at the person's best time of day.

#121 · Assessment

How do delirium, dementia and depression differ on onset, course and attention?

Answer

Delirium is acute and fluctuating, impairs attention early, and is a medical emergency with an underlying cause. Dementia is chronic and progressive with attention preserved early. Depression can mimic both, which is why every positive mood screen gets a medical differential. Slowed processing with age is normal, losing medication or finance management is not.

#122 · Assessment

Is universal intimate partner violence screening recommended in Canada, and what is the practical standard?

Answer

The Canadian Task Force on Preventive Health Care found the evidence insufficient for universal routine screening, so the standard is case finding. Recognize indicators such as injuries inconsistent with the stated mechanism, delayed presentation, chronic pain, and a controlling partner who will not leave the room, then ask directly, in private, with an interpreter who is not family.

#123 · Assessment

A client discloses intimate partner violence. What is the correct assessment response?

Answer

Do not direct them to leave. Assess immediate danger by asking about weapons, threats to kill, strangulation and stalking, since non fatal strangulation strongly predicts later lethality. Then safety plan, document in the person's own words, and offer referral. An adult with capacity decides for themselves whether to involve police.

#124 · Assessment

What is the shared core of the duty to report a child in need of protection across Canada?

Answer

Every province and territory imposes a legal duty on professionals. The duty is personal and cannot be delegated to a supervisor or social worker, it is triggered by reasonable grounds to suspect rather than by proof, and it is ongoing, so a new suspicion requires a new report.

#125 · Assessment

Which parts of the child protection reporting duty vary between Canadian jurisdictions?

Answer

The upper age and the receiving body. Ontario's mandatory duty applies under 16, with a discretionary duty for 16 and 17 year olds. Reports go to a children's aid society or Indigenous child and family well being agency in Ontario, a Director of Child Welfare elsewhere, and the Director of Youth Protection in Quebec.

#126 · Assessment

How do elder abuse reporting duties vary across Canada?

Answer

Far more than for children. Nova Scotia, Newfoundland and Labrador, and Prince Edward Island have adult protection legislation with broad mandatory reporting. Several provinces mandate reporting only in licensed care settings, such as Alberta's Protection for Persons in Care Act. Most jurisdictions have no blanket duty for a capable older adult in the community.

#127 · Assessment

With a capable community dwelling older adult you suspect is being abused, what do you assess and screen for?

Answer

Assess capacity, address immediate safety, document, and support the person's own decisions rather than overriding them. Screen for financial abuse, the most common and least visible form, and assess caregiver strain, since burnout and abuse often share a household.

#128 · Assessment

Name the four elements of trauma and violence informed care and how they show up in practice.

Answer

Awareness of trauma and violence, safety and trustworthiness, choice and collaboration, and a strengths based approach. In practice: explain before touching, offer choices about positioning and who is present, ask permission before sensitive examination, and shift the question from what is wrong with you to what happened to you.

#129 · Assessment

What produces intergenerational trauma in Canada, and who judges whether care was culturally safe?

Answer

Residential schools, the Sixties Scoop, forced relocation, the Indian Act and ongoing systemic racism, with effects passing across generations through disrupted parenting, family separation, cultural loss and chronic stress. Cultural safety is judged by the person receiving care, not the provider, so name racism as a determinant rather than reading an outcome as non compliance.

#130 · Assessment

How do you screen for food insecurity, and what obligation does asking create?

Answer

Ask whether in the past 12 months the food the household bought did not last and there was no money to get more. Asking creates an obligation to connect the person to something real, such as a community health centre, income support navigation, or a food program. Screening without linkage adds burden and yields nothing.

#131 · Diagnosis

What is hypothetico deductive reasoning, and what makes a question or test worth asking?

Answer

The deliberate method: generate a small set of candidate explanations early, then gather history, findings and investigations that discriminate between them. A question or test that would answer the same way under every hypothesis adds nothing and should not be asked.

#132 · Diagnosis

What is pattern recognition in diagnosis, and what mental structure does it rely on?

Answer

The fast method. Experienced clinicians store illness scripts, mental templates holding a disease's typical predisposing conditions, pathophysiology and clinical consequences. When a presentation matches a stored script closely, recognition happens before any conscious analysis.

#133 · Diagnosis

Define System 1 and System 2 under dual process theory, and name the real failure mode.

Answer

System 1 is fast, intuitive and effortless, running on pattern recognition. System 2 is slow, analytic and effortful, running on hypothetico deductive reasoning. System 1 is accurate most of the time. The failure is not using System 1, it is missing the moment to switch to System 2.

#134 · Diagnosis

Name five triggers to switch deliberately from System 1 to System 2, and the habit that enables it.

Answer

Switch when the presentation is atypical for the script it matches, when the client is not improving as the working diagnosis predicts, when you feel rushed, when someone else labelled the client first, or when the stakes of being wrong are high. The diagnostic pause, a ten second stop before committing, is the highest yield habit.

#135 · Diagnosis

What is anchoring bias, and what is its countermeasure?

Answer

Fixing on an early feature and failing to adjust as new information arrives. A client says their asthma is flaring, so raised jugular venous pressure and bibasilar crackles get filed as incidental instead of heart failure. Countermeasure: name your anchor, ask what finding would move you off it, then check for it.

#136 · Diagnosis

What is premature closure, and what two questions prevent it?

Answer

Stopping the search once a plausible answer appears, such as calling vertigo in a 70 year old benign paroxysmal positional vertigo after a positive Dix Hallpike and never exploring the new unsteady gait. Before closing, ask what else this could be and what the worst thing it could be is.

#137 · Diagnosis

What is availability bias, and what replaces a felt sense of probability?

Answer

Judging likelihood by how easily an example comes to mind. After two pulmonary emboli in a month, the next pleuritic chest pain feels like a third.

Countermeasure: use an explicit rule such as the Wells or PERC criteria rather than an intuition about how likely the diagnosis feels.

#138 · Diagnosis

What is confirmation bias in diagnostic reasoning, and how do you counter it?

Answer

Weighting evidence that supports your hypothesis while discounting evidence against it. You believe the fatigue is depression, so a normal PHQ-9 is called an underreport and a low ferritin is called incidental. Countermeasure: hunt for the finding that would disconfirm you, and weight it equally.

#139 · Diagnosis

What is the framing effect, and what neutralizes it?

Answer

Being swayed by how the problem is presented. A triage note reading frequent flyer, anxiety raises your threshold for investigating the tachycardia. Countermeasure: restate the case in neutral language, stripping out the labels others attached, then reassess the findings on their own.

#140 · Diagnosis

What is search satisficing, and what rule prevents it?

Answer

Calling off the search after the first abnormality. The chest radiograph shows a rib fracture and the small apical pneumothorax is never reported. Countermeasure: on any positive result, finish the whole review rather than stopping at the finding that explains the symptom.

#141 · Diagnosis

What is diagnostic momentum, and what question should you ask about any inherited diagnosis?

Answer

A label from an earlier clinician becoming harder to question over time. A client carries COPD for four years, spirometry was never done, and the real problem is heart failure. Ask what objective evidence established the diagnosis and whether that evidence is actually in the record.

#142 · Diagnosis

What is fundamental attribution error in clinical reasoning, and what do you ask before concluding nonadherence?

Answer

Blaming the person rather than the situation, such as calling recurrent hyperglycemia nonadherence when the client cannot afford the insulin. Ask about cost, access, housing and understanding before concluding a client chose not to follow the plan.

#143 · Diagnosis

What single structure counters all eight cognitive biases described in Chapter 4?

Answer

Slow down, name the working diagnosis out loud, and set a condition that would change your mind. Diagnostic error in primary care is more often a thinking failure than a knowledge failure, so the countermeasure is a habit of reasoning rather than more facts.

#144 · Diagnosis

What three axes structure a differential diagnosis, and what does each one ask?

Answer

Most likely asks what the base rates say. Most dangerous asks what would harm this person if missed today. Most treatable asks where the answer changes what you can do. A differential is a short, ordered, decision ready set, not a list of everything a symptom can cause.

#145 · Diagnosis

List the must not miss diagnoses for undifferentiated chest pain.

Answer

Acute coronary syndrome, pulmonary embolism, aortic dissection, tension pneumothorax, and pericarditis with tamponade. These earn a place on the differential even at low probability, because what counts is probability multiplied by consequence, not probability alone.

#146 · Diagnosis

List the must not miss diagnoses for headache and for back pain.

Answer

Headache: subarachnoid hemorrhage, meningitis, giant cell arteritis, raised intracranial pressure. Back pain: cauda equina syndrome, spinal infection, malignancy, abdominal aortic aneurysm. Each stays on the list at low probability because the cost of missing it today is severe.

#147 · Diagnosis

Why do hypothyroidism, B12 deficiency and iron deficiency sit high on a fatigue differential?

Answer

They belong to the most treatable axis rather than the most likely one. They are cheap to detect and fully correctable, so finding them changes what you can do. Medication adverse effects belong in the same group and deserve the same early consideration.

#148 · Diagnosis

How long should a written differential be, and what single test must justify itself?

Answer

Three to five ranked entries plus the must not miss items. Then choose investigations that separate them, and for each test say what result would change your management. If no result would change management, the test is not indicated.

#149 · Diagnosis

Name five Choosing Wisely Canada recommendations against routine testing, and the harms of overtesting.

Answer

Against routine annual blood work in asymptomatic adults, routine vitamin D testing, thyroid testing in asymptomatic low risk people, imaging for low back pain without red flags, and routine ECGs in low risk asymptomatic clients. Overtesting generates false positives, incidental findings, cascade investigations, cost and anxiety.

#150 · Diagnosis

Define sensitivity and specificity, and state whether they change between settings.

Answer

Sensitivity is the proportion of people with the condition whom the test calls positive. Specificity is the proportion without the condition whom it calls negative. Both are properties of the test itself and stay roughly stable across settings and populations.

#151 · Diagnosis

Define positive and negative predictive value, and state what they depend on.

Answer

Positive predictive value is the proportion of positive results that are true, and negative predictive value is the proportion of negative results that are true. Both depend heavily on how common the condition is in the population being tested, which is the concept most often misunderstood.

#152 · Diagnosis

A 90 percent sensitive and 90 percent specific test is run where prevalence is 1 percent. What is the PPV?

Answer

Of 10,000 people, 100 have the condition: 90 true positives and 10 missed. Of the 9,900 without it, 10 percent test positive, giving 990 false positives. PPV is 90 divided by 1,080, which is 8.3 percent, so more than nine of every ten positives are wrong.

#153 · Diagnosis

The same 90 and 90 test is run where prevalence is 30 percent. What is the PPV, and what is the lesson?

Answer

3,000 have the condition and 7,000 do not, giving 2,700 true positives and 700 false positives. PPV is 2,700 divided by 3,400, which is 79.4 percent. Same test, meaning of a positive moved from 8.3 to 79.4 percent purely on prevalence.

#154 · Diagnosis

How is the positive likelihood ratio calculated, and what does it equal for a 90 and 90 test?

Answer

Positive likelihood ratio is sensitivity divided by one minus specificity. For a test with 90 percent sensitivity and 90 percent specificity, that is 0.90 divided by 0.10, which equals 9. It multiplies the pre test odds to give the post test odds.

#155 · Diagnosis

How is the negative likelihood ratio calculated, and what does it equal for a 90 and 90 test?

Answer

Negative likelihood ratio is one minus sensitivity divided by specificity. For a 90 percent sensitive and 90 percent specific test that is 0.10 divided by 0.90, which equals 0.11. A small negative likelihood ratio means a negative result meaningfully lowers probability.

#156 · Diagnosis

Which likelihood ratio values usually change management, and which carry no information?

Answer

A likelihood ratio above 10 or below 0.1 usually changes management. A ratio between 0.5 and 2 usually does not. A ratio of exactly 1 carries no information at all, because the result is equally likely with or without the condition.

#157 · Diagnosis

Why does a negative D dimer rule out pulmonary embolism at 10 percent pre test probability but not at 60 percent?

Answer

With sensitivity near 95 percent and specificity near 40 percent, the negative likelihood ratio is 0.125. From 10 percent, post test probability falls to about 1.4 percent, below the testing threshold. From 60 percent, roughly 16 percent residual risk remains, so that client goes straight to imaging.

#158 · Diagnosis

State adult reference intervals for hemoglobin and platelets in SI units, and the first step in classifying anemia.

Answer

Hemoglobin roughly 120 to 160 g/L in females and 135 to 175 g/L in males, platelets 150 to 400 $\times 10^9/L$. Classify anemia by mean corpuscular volume first: below 80 fL suggests iron deficiency or thalassemia trait, above 100 fL suggests B12 or folate deficiency, alcohol, hypothyroidism or medications.

#159 · Diagnosis

What diagnosis belongs on the differential for any new anemia in an adult over 50?

Answer

Colorectal malignancy, until it is excluded. Normocytic anemia also points to anemia of chronic disease, acute blood loss or renal disease, but age over 50 with new anemia makes gastrointestinal malignancy a must not miss entry regardless of the cell size.

#160 · Diagnosis

How is the anion gap calculated, and what does a value of 28 mean?

Answer

Anion gap is sodium minus the sum of chloride and bicarbonate. With sodium 138 mmol/L, chloride 98 mmol/L and bicarbonate 12 mmol/L, the gap is 28, indicating a high anion gap metabolic acidosis. Normal sodium is 135 to 145 and bicarbonate 22 to 29 mmol/L.

#161 · Diagnosis

How do you correct calcium for a low albumin, and is 2.10 mmol/L with albumin 28 g/L abnormal?

Answer

Add 0.02 mmol/L for every 1 g/L that albumin falls below 40 g/L. Albumin 28 g/L is 12 below, so add 0.24. Corrected calcium is 2.10 plus 0.24, which is 2.34 mmol/L, and is therefore normal despite the low reported value.

#162 · Diagnosis

State the CKD G categories by eGFR in mL/min/1.73m².

Answer

G1 is 90 or above, G2 is 60 to 89, G3a is 45 to 59, G3b is 30 to 44, G4 is 15 to 29, and G5 is below 15. Canadian laboratories report eGFR automatically with every creatinine using the CKD-EPI equation without a race coefficient.

#163 · Diagnosis

State the albuminuria categories in mg/mmol, and why one low eGFR is not a CKD diagnosis.

Answer

Urine albumin to creatinine ratio: A1 below 3, A2 from 3 to 30, A3 above 30 mg/mmol. CKD requires the abnormality to persist at least three months, so a single low eGFR is not a diagnosis, and G1 or G2 without albuminuria or other kidney damage is not CKD.

#164 · Diagnosis

Distinguish hepatocellular from cholestatic liver enzyme patterns, and which tests measure synthetic function.

Answer

A hepatocellular pattern raises ALT and AST disproportionately; a cholestatic pattern raises alkaline phosphatase and gamma glutamyl transferase. Bilirubin runs roughly 3 to 20 micromol/L. Albumin, INR and bilirubin measure synthetic function while ALT and ALP measure injury, so enzymes can be normal while function fails.

#165 · Diagnosis

Interpret the three main TSH and free T4 combinations, and when to repeat rather than treat.

Answer

Raised TSH with low free T4 is primary hypothyroidism. Raised TSH with normal free T4 is subclinical hypothyroidism, usually warranting a repeat in six to eight weeks rather than immediate treatment. Suppressed TSH with raised free T4 is hyperthyroidism. Test TSH first.

#166 · Diagnosis

With total cholesterol 6.2, HDL 1.1 and triglycerides 1.76 mmol/L, calculate LDL and non HDL.

Answer

Friedewald LDL is $6.2 - 1.1 - 0.8$, where 0.8 is $1.76 \div 2.2$, giving 4.3 mmol/L. Non HDL is $6.2 - 1.1$, giving 5.1 mmol/L. Fasting is no longer required for routine screening, and apolipoprotein B is an accepted Canadian alternative marker.

#167 · Diagnosis

State the Diabetes Canada diagnostic thresholds and the prediabetes range.

Answer

Diabetes: HbA1c 6.5 percent or above, fasting plasma glucose 7.0 mmol/L or above, two hour glucose on a 75 g oral glucose tolerance test 11.1 mmol/L or above, or random glucose 11.1 mmol/L or above. Prediabetes is HbA1c 6.0 to 6.4 percent or fasting glucose 6.1 to 6.9 mmol/L. Asymptomatic clients need a confirmatory repeat.

#168 · Diagnosis

In which four situations is HbA1c unreliable for assessing glycemia?

Answer

When red cell turnover is abnormal: hemoglobinopathies, recent transfusion, iron deficiency, and advanced chronic kidney disease. In these situations use plasma glucose measurements instead, because the HbA1c value no longer reflects average glycemia over the preceding months.

#169 · Diagnosis

State the ferritin, B12 and INR values that guide interpretation in Canadian units.

Answer

Ferritin below 30 micrograms/L confirms iron deficiency, but as an acute phase reactant a value up to 100 can mask deficiency during inflammation, and transferrin saturation below 20 percent supports it. B12 below roughly 150 pmol/L is deficient; 150 to 220 pmol/L is clarified by methylmalonic acid. INR near 1.0 is normal and warfarin targets 2.0 to 3.0.

#170 · Diagnosis

Why is a single high office blood pressure reading never a diagnosis of hypertension?

Answer

The 2025 Hypertension Canada diagnostic threshold is a blood pressure of 130/80 mmHg or higher, and white coat elevation is common. Confirm with a validated device under optimal conditions, using out of office measurement such as home readings or ambulatory monitoring, before labelling anyone hypertensive.

#171 · Diagnosis

State the fixed reading order for an ECG, and calculate the QTc for a QT of 440 ms at rate 75.

Answer

Read rate, rhythm, axis, intervals, then chambers and ST segments. Rate is 300 divided by the number of large squares between R waves. Normal PR is 120 to 200 ms and QRS under 120 ms. At rate 75 the RR is 0.8 seconds, square root 0.894, so Bazett QTc is about 492 ms, prolonged, demanding a medication review.

#172 · Diagnosis

Name the laboratory results that demand same day action however well the client looks.

Answer

Potassium below 2.5 or above 6.0 mmol/L, sodium below 120 or above 155 mmol/L, corrected calcium below 1.75 or above 3.0 mmol/L, hemoglobin below 70 g/L, platelets below $20 \times 10^9/L$, neutrophils below $0.5 \times 10^9/L$, glucose below 3.0 or above 25 mmol/L, INR above 5.0, any positive blood culture, and any eGFR fallen by over 25 percent.

#173 · Diagnosis

What are the four features of a closed loop test result system?

Answer

Every ordered test is tracked to a result, every result is acknowledged by a named clinician, every abnormal result generates a documented action and a documented message to the client, and normal results are reported too, because no news is good news fails when a result never arrives. Locum coverage needs explicit handover of pending results.

#174 · Diagnosis

What does teach back actually require, and what does SPIKES stand for?

Answer

Teach back asks the client to explain in their own words what the problem is and what happens next, showing what actually transferred. SPIKES is Setting, Perception, Invitation, Knowledge, Emotion, Summary. Respond to emotion before adding facts, and treat silence after bad news as a clinical intervention.

#175 · Diagnosis

What makes advice genuine safety netting, and how does planned reassessment help?

Answer

Safety netting names specific findings, a time frame, and a destination. Telling a client to return if worse is not safety netting. Planned reassessment turns time into a diagnostic test: a 48 hour recheck for undifferentiated abdominal pain lets the illness declare itself under supervision instead of at home.

#176 · Management

When moving from diagnosis to care plan, what do you present to the client first?

Answer

Options, not a decision. For most conditions there is more than one reasonable path: watchful waiting, non pharmacological management, medication, referral, or a combination. Say what each involves, what it achieves, what it costs in money and effort, and what happens if the client does nothing. Doing nothing for now is a legitimate option, named out loud.

#177 · Management

What four elements must you demonstrate for valid informed consent?

Answer

The client has capacity for this specific decision at this time. The client received the nature of the treatment, the expected benefits, the material risks, the reasonable alternatives, and the consequences of declining. The consent is voluntary. The consent is specific to what you are proposing.

#178 · Management

Why is capacity described as decision specific, and what happens when a client lacks it?

Answer

A client may be capable of consenting to an antibiotic and incapable of consenting to a complex surgical decision the same day. If capacity is absent, move to the substitute decision maker hierarchy in your province or territory, and ask that person to decide on the client's prior expressed wishes first, then best interests.

#179 · Management

How do client goals shape the plan, and what makes an answer option wrong on the exam?

Answer

Ask what matters most to the client and what a good result looks like, then record the answer in their own words. A client whose goal is to keep working night shifts needs a different diabetes plan than one chasing the lowest A1C. An option matching guideline targets but ignoring a stated client goal is usually wrong.

#180 · Management

What four questions decide whether a care plan is feasible?

Answer

Is it available, given backorders, wait times and travel distance. What does it cost the client. Do the determinants of health support it. Is it effective and likely to be followed. A plan the client cannot afford, cannot get to, or cannot keep up is not a plan.

#181 · Management

How does drug coverage vary in Canada, and which federal programs have their own formularies?

Answer

Canada has no single national drug plan. Coverage depends on province or territory, age, income and private workplace plans, so a drug covered in one province may need special authority or exception status in another. Non Insured Health Benefits for First Nations and Inuit clients and the Interim Federal Health Program for refugee claimants run their own formularies.

#182 · Management

Which determinants of health shape adherence, and how should a failed plan be framed?

Answer

Housing, food security, income, literacy, transportation, caregiving load, immigration status and access to refrigeration. Twice daily insulin is not sustainable for a client with no fixed address and no fridge. That is a plan problem, not a client problem.

#183 • Management

How do you weigh effectiveness against treatment burden when choosing a regimen?

Answer

Weigh the number needed to treat against what the regimen demands of the client's day. A five dose daily regimen loses to a once daily one in almost every real comparison, so the more effective drug on paper can be the less effective plan in practice.

#184 • Management

State the CNPLE prioritization order, and apply it to four simultaneous problems.

Answer

Clinical urgency first, since anything threatening life, limb or sight comes before everything else. Then the client's own stated priority. Then what is feasible with available resources. Last, guideline items that can safely wait. With new hypertension, untreated depression, food insecurity and an A1C of 8.4 percent, ask which problem is hardest right now rather than writing four prescriptions.

#185 · Management

Match each common barrier to its practical solution.

Answer

Cost: formulary alternatives, special authority applications, compassionate access programs, or larger dispensing intervals. Transportation: virtual follow up, community paramedicine, or grouping appointments on one day. Health literacy: teach back and plain language instructions. Language: a professional interpreter, never a family member and never a child.

#186 · Management

What does consultation mean, and who remains the most responsible provider?

Answer

You keep responsibility for the client and ask a colleague for advice on a specific question, remaining the most responsible provider. Consult when the diagnosis is uncertain, when the client is not responding as expected, when treatment sits at the edge of your competence, or when a second opinion would change your plan.

#187 · Management

What does referral mean, and what accountability stays with you?

Answer

You send the client to another provider for a defined problem while you continue managing everything else. You stay accountable for follow up until the referral is accepted and acted upon. A referral that is never acknowledged is still yours to chase.

#188 · Management

What does transfer of care mean, and what two things must happen for it to be valid?

Answer

Another provider assumes responsibility for the client, or for that episode, and you step out. Transfer requires the receiving provider to accept, and a clear handover. Without acceptance and handover, responsibility has not actually moved anywhere.

#189 · Management

What belongs in a safe consultation, referral or transfer communication?

Answer

The clinical question in one sentence, the working diagnosis, relevant history, medications, allergies, investigations already done with results and dates, and what you have tried. State urgency concretely, such as needs assessment within 48 hours, rather than the word urgent. Tell the client what was sent, to whom, and what to do if nothing is heard by a stated date.

#190 · Management

Is nurse practitioner authority to order imaging, admit or refer set nationally?

Answer

No. Authority to order specific imaging, to admit, to complete certain forms and to refer directly to some specialties is set provincially and by employer policy. Knowing your own jurisdiction's limits is part of safe practice, and the exam expects that rather than an assumed national standard.

#191 · Management

What blood pressure defines hypertension in adults under the 2025 Hypertension Canada guideline?

Answer

A blood pressure of 130/80 mmHg or higher, measured with a validated device under optimal conditions. This replaced the previous 140/90 mmHg standard, so a group of people who were not called hypertensive before now are. Confirm properly before labelling anyone, because white coat hypertension is common.

#192 · Management

State the 2025 Hypertension Canada treatment target, and how it differs from the superseded one.

Answer

A systolic blood pressure below 130 mmHg, provided treatment is well tolerated. The 2025 guideline sets a systolic target only and deliberately sets no diastolic treatment target, a real departure from the older 140/90 style paired target. A candidate answering with a paired target is answering from the superseded guideline.

#193 • Management

Which adults should start antihypertensive pharmacotherapy under the 2025 guideline?

Answer

Adults with a blood pressure of 140/90 mmHg or higher, and also adults with a systolic pressure of 130 to 139 mmHg who are at high cardiovascular risk. Cardiovascular risk, not the number alone, decides who starts a drug in that middle band.

#194 • Management

What is recommended for systolic pressure of 130 to 139 mmHg without high cardiovascular risk?

Answer

Healthy lifestyle change alone, with blood pressure reassessed within three to six months. No antihypertensive drug is started in that band unless cardiovascular risk is high. Sodium reduction, weight management and exercise are the first line intervention here.

#195 · Management

When should a blood pressure target be individualized, and what suggests a secondary cause?

Answer

In frail older adults, a fall caused by orthostatic hypotension can do more harm than the few millimetres of mercury gained, so tolerability governs. Suspect secondary hypertension with onset under 30, sudden loss of control, hypokalemia, or resistance to three drugs including a diuretic.

#196 · Management

State the Diabetes Canada diagnostic criteria for type 2 diabetes and the confirmation rule.

Answer

Fasting plasma glucose 7.0 mmol/L or higher, A1C 6.5 percent or higher, two hour plasma glucose 11.1 mmol/L or higher on a 75 g oral glucose tolerance test, or random plasma glucose 11.1 mmol/L or higher. A single abnormal test in an asymptomatic person needs a repeat confirmatory test.

#197 · Management

What is the usual A1C target in type 2 diabetes, and when is it relaxed?

Answer

7.0 percent or less for most adults. Diabetes Canada relaxes it for older adults to 7.1 to 8.0 percent when functionally dependent and 7.1 to 8.5 percent with frailty or dementia, and where hypoglycemia unawareness make tight control dangerous. The relaxed target is a safety decision about hypoglycemia risk, not a concession to poor control.

#198 · Management

Metformin is first line in type 2 diabetes, so what decides the second agent?

Answer

Comorbidity. With cardiovascular disease, heart failure, or chronic kidney disease, choose an agent with proven organ protection rather than one that only lowers glucose. The decision point on the exam is the comorbid condition in the stem, not the size of the A1C gap.

#199 · Management

What four assessments make up annual screening in type 2 diabetes?

Answer

Urine albumin to creatinine ratio, eGFR, a dilated eye exam, and a monofilament foot exam. These detect the microvascular complications that are silent until late, and they are performed on a scheduled annual basis rather than only when symptoms appear.

#200 · Management

What ACR value is abnormal in chronic kidney disease, and what is foundational therapy?

Answer

An albumin to creatinine ratio of 3 mg/mmol or higher is abnormal and needs confirmation on a repeat sample. Blood pressure control plus an ACE inhibitor or an angiotensin receptor blocker in albuminuric disease is foundational. Stage by eGFR and by ACR together, not by eGFR alone.

#201 · Management

List the criteria for nephrology referral in chronic kidney disease.

Answer

eGFR persistently below 30 mL/min/1.73 m², ACR above 60 mg/mmol, rapid unexplained decline in kidney function, refractory hyperkalemia, or suspected glomerular disease. Any one of these warrants referral rather than continued primary care monitoring alone.

#202 · Management

Which statin indicated conditions bypass cardiovascular risk calculation entirely?

Answer

Clinical atherosclerotic disease, abdominal aortic aneurysm, chronic kidney disease, most diabetes over defined age or duration thresholds, and an LDL cholesterol of 5.0 mmol/L or higher. For everyone else, calculate a Framingham based risk score and discuss the result with the client.

#203 • Management

State the Canadian secondary prevention intensification thresholds for lipids.

Answer

LDL cholesterol at or above 1.8 mmol/L, non HDL cholesterol at or above 2.4 mmol/L, or apolipoprotein B at or above 0.7 g/L. Reaching any one of these thresholds on treatment prompts intensification of lipid lowering therapy in a client with established disease.

#204 • Management

How is heart failure with reduced ejection fraction managed, and what does NT proBNP add?

Answer

The Canadian Cardiovascular Society approach uses four foundational drug classes started early and titrated together rather than one at a time to maximum dose. NT proBNP helps rule out heart failure in undifferentiated dyspnea. Daily weights, sodium and fluid guidance, and a written action plan prevent most admissions.

#205 · Management

How does the Canadian Thoracic Society define asthma control, and what precedes escalating therapy?

Answer

Control is defined by symptom frequency, night waking, reliever use, activity limitation, absence from work or school, and lung function. Poor control triggers a review of adherence, inhaler technique and the diagnosis itself before escalating therapy. Check technique at every visit, because much apparent treatment failure is device failure.

#206 · Management

What confirms COPD, and which interventions change outcomes most?

Answer

Post bronchodilator spirometry showing an FEV1 to FVC ratio below 0.7. Management follows symptom burden and exacerbation history, not FEV1 alone. Pulmonary rehabilitation, smoking cessation and vaccination change outcomes more than most inhaler adjustments.

#207 · Management

What is the base of every osteoarthritis plan, and what triggers a surgical opinion?

Answer

Exercise, education and weight management form the base of every plan. Topical anti inflammatories for knee and hand disease are safer than oral agents in older adults. Refer for surgical opinion on failed conservative care, not on radiographic severity.

#208 · Management

How is hypothyroidism diagnosed and monitored, and why wait six to eight weeks?

Answer

Diagnose with TSH, adding free T4 when TSH is abnormal. Recheck TSH six to eight weeks after any dose change, because earlier testing reflects a level that has not stabilized. Subclinical disease usually deserves observation rather than immediate treatment.

#209 · Management

How do the Canadian Adult Obesity Clinical Practice Guidelines reframe obesity, and how should the topic be raised?

Answer

As a chronic disease defined by health impairment rather than BMI alone. Ask permission before discussing weight, assess root causes and functional impact, agree on health goals rather than weight goals, and address weight bias in your own practice.

#210 · Management

What is the nurse practitioner's value in an uncomplicated upper respiratory infection?

Answer

Most are viral. Your value is ruling out pneumonia, considering a delayed prescription rather than an immediate antibiotic, and giving a clear return precaution that names specific findings and a time frame.

#211 · Management

How does the modified Centor score direct pharyngitis management, and why is penicillin still first line?

Answer

Low scores need neither testing nor antibiotics. Intermediate scores are swabbed and treated only if the swab is positive. Penicillin remains first line because group A streptococcal resistance to penicillin has not emerged.

#212 · Management

When is watchful waiting appropriate in acute otitis media, and when do you treat promptly?

Answer

Watchful waiting with analgesia and a safety net prescription suits a healthy child six months or older with non severe unilateral disease and reliable follow up. Treat promptly with severe pain, high fever, perforation, or bilateral disease in the very young.

#213 · Management

When is asymptomatic bacteriuria treated, and what does cloudy urine in an older adult mean?

Answer

Only in pregnancy and before urologic procedures breaching the mucosa. Cloudy or strong smelling urine in an older adult without urinary symptoms is not an infection and does not warrant an antibiotic. Treat symptomatic cystitis with a short course agent guided by local resistance.

#214 · Management

How do you track cellulitis progression, and what findings mean emergency transfer?

Answer

Mark the border with a pen and date it, because progression is the clearest measure of treatment failure. Pain out of proportion to findings, rapid spread, crepitus, bullae, skin necrosis, or systemic toxicity suggest necrotizing infection and need emergency transfer, not an oral antibiotic.

#215 · Management

What is the main intervention in gastroenteritis, and when are antimotility agents avoided?

Answer

Oral rehydration is the intervention. Antibiotics are rarely indicated. Avoid antimotility agents when there is bloody diarrhea or high fever, since those features point to an invasive process that slowing the bowel would worsen.

#216 · Management

What red flags are screened in low back pain, and which Canadian rules reduce imaging safely?

Answer

Cauda equina symptoms, suspected fracture, malignancy and infection. Without red flags, imaging in the first six weeks changes nothing and Choosing Wisely Canada advises against it, so keep the client moving. The Ottawa ankle and knee rules and the Canadian C spine rule are validated tools that safely reduce imaging in minor injury.

#217 · Management

What is the very first action in anaphylaxis, and at what site, concentration and dose?

Answer

Intramuscular epinephrine, given before calling for help, before an antihistamine, before a corticosteroid, and before any airway maneuver other than positioning. Inject into the anterolateral thigh using the 1 mg/mL concentration, 0.01 mg/kg, to a usual maximum of 0.5 mg in an adult and 0.3 mg in a child.

#218 · Management

After the first dose of epinephrine in anaphylaxis, what is the order of the next steps?

Answer

Call for emergency help, lay the client supine with legs elevated unless vomiting or in respiratory distress, give high flow oxygen, obtain intravenous access, and give a fluid bolus for hypotension. Repeat epinephrine every 5 to 15 minutes if the response is inadequate.

#219 · Management

Why is giving an antihistamine or corticosteroid first in anaphylaxis a fatal error, and what follow up is needed?

Answer

Antihistamines and corticosteroids treat neither the airway nor the shock, so time spent on them is time the client is not receiving the only drug that works. Afterwards observe for biphasic reactions, prescribe two auto injectors, teach their use, and refer to allergy.

#220 · Management

How is sepsis recognized, and what must happen within the first hour?

Answer

Suspect it in any client with infection plus new organ dysfunction, altered mental state, hypotension, or a raised lactate. Within the first hour: obtain blood cultures before antibiotics only if this causes no delay, give broad spectrum antibiotics, measure lactate, and give a crystalloid bolus for hypotension or a lactate of 4 mmol/L or higher. Arrange emergency transfer.

#221 · Management

State the first steps in suspected acute coronary syndrome, and when nitroglycerin is withheld.

Answer

Obtain a 12 lead ECG within 10 minutes of first contact, give chewed non enteric coated acetylsalicylic acid unless truly contraindicated, call emergency services, and give oxygen only if saturation is low. Withhold nitroglycerin with hypotension, suspected right ventricular infarction, or recent phosphodiesterase inhibitor use. A STEMI transfer decision turns on time to catheterization, not symptom relief.

#222 · Management

What are the immediate priorities in suspected acute stroke?

Answer

Establish the last known well time, because it drives treatment eligibility. Check capillary glucose immediately, since hypoglycemia mimics stroke. Activate the stroke pathway and arrange emergency imaging. Do not lower blood pressure routinely in the acute phase, and give nothing by mouth until swallowing is screened.

#223 • Management

How is a severe asthma exacerbation treated, and what signals imminent respiratory failure?

Answer

High dose inhaled short acting bronchodilator with an anticholinergic, oxygen titrated to target saturation, and a systemic corticosteroid given early. Silent chest, exhaustion, drowsiness, or a rising carbon dioxide level signal imminent respiratory failure and immediate transfer.

#224 • Management

Define hypoglycemia in Canada and give the treatment for a conscious and an unconscious client.

Answer

A plasma glucose below 4.0 mmol/L. If conscious and able to swallow, give 15 g of fast acting carbohydrate, recheck in 15 minutes, repeat if still low, then give a snack or meal. If unconscious or unable to swallow, put nothing in the mouth: give intravenous dextrose, or glucagon if no line is available, then reassess and address the cause.

#225 · Management

What comes before naloxone in opioid overdose, and why must you stay with the client?

Answer

Airway and breathing, because the client dies of hypoventilation. Check response, call emergency services, open the airway, give rescue breaths or bag mask ventilation with oxygen, then give naloxone and repeat every 2 to 3 minutes until breathing is adequate. Stay, because naloxone wears off before many opioids do. The Good Samaritan Drug Overdose Act protects bystanders who call.

#226 · Management

How do you approach acute psychosis with risk, and which medical causes must be excluded?

Answer

Safety of the client, staff and others comes before diagnosis. Reduce stimulation, keep an exit path, use a calm single speaker, and attempt verbal de escalation before any restraint. Rule out hypoglycemia, intoxication, withdrawal, infection and delirium. Involuntary assessment forms exist under provincial mental health legislation, and NP authority to complete them differs by jurisdiction.

#227 · Management

What must be confirmed before a procedure, and what must be provided afterward?

Answer

Beforehand: the indication, the alternatives including doing nothing, the material risks in plain language, documented consent, plus allergies, anticoagulant use and pregnancy status where relevant. Afterward: written aftercare, the signs of complication, who to call and when, pathology follow up with a defined date, and documentation of technique, specimen, anesthetic, tolerance and plan.

#228 · Management

When does palliative care start, and how is a goals of care conversation structured?

Answer

At diagnosis of a life limiting illness, not at the end, introduced as an extra layer of support. Ask what the client already understands, ask permission to share information, give it plainly, stop and respond to emotion, then explore what matters most and translate that into orders. Document the goals and revisit them when the illness changes.

#229 · Management

Name the palliative symptom control priorities, and explain why opioids are not hastening death.

Answer

Pain, dyspnea, nausea, secretions, constipation and delirium. Opioids relieve both pain and breathlessness, and titration against symptoms in the dying client is neither hastening death nor sedation for its own sake. Prescribe ahead for the crisis that is coming, because a symptom kit in the home prevents a distressing transfer.

#230 · Management

What is the nurse practitioner's role in medical assistance in dying in Canada?

Answer

NPs may act as assessors and providers under federal Criminal Code criteria, but provincial law, regulator standards and employer policy modify what an individual NP may do and where. Requests must come from the client. Explore what drives the request, treat reversible suffering, and give accurate information. A conscientious objector may not abandon the client and must not obstruct access.

#231 • Management

Who sets nurse practitioner prescribing authority in Canada, and what does that mean for an exam stem?

Answer

Each province and territory through its nursing regulator, not the federal government. Some jurisdictions authorize the full drug schedule while others retain limits on specific classes. Where a stem names no province, confirm the local standard rather than exporting one province's rule nationally.

#232 • Management

List the required elements of a valid written prescription in Canada.

Answer

Client's full name plus a second identifier such as date of birth, the date written, drug name, strength, dosage form, quantity or duration of supply, complete directions including route and frequency, the number of repeats or a statement that there are none, and the prescriber's name, designation, registration number, practice contact information and signature.

#233 · Management

Why are 5.0 mg and .5 mg both unsafe ways to write a dose?

Answer

A trailing zero invites a tenfold overdose if the decimal point is not seen, and a naked decimal without a leading zero does exactly the same. Write 5 mg and 0.5 mg instead, so the magnitude survives a smudge, a fax, or a hurried read.

#234 · Management

Which prescription abbreviations cause documented misreadings, and why is take as directed unsafe?

Answer

U for units and QD for daily both produce documented misreadings and should be written out. Take as directed hands your clinical decision to a pharmacist who was not in the room, so the directions must state route, dose and frequency in full.

#235 · Management

When is a verbal order appropriate, and what steps make it safe?

Answer

Only when writing is genuinely impossible, such as during a resuscitation. The receiver writes the order down, reads it back digit by digit, the prescriber confirms, and it is countersigned within the organization's timeframe. Verbal ordering is a controlled exception, not a convenience.

#236 · Management

What are the risks of faxed prescriptions, and what is PrescribeIT?

Answer

Faxes remain legal in most jurisdictions but can be intercepted, misdirected, or altered. PrescribeIT, operated by Canada Health Infoway, transmits a prescription from the prescriber's electronic medical record straight into the pharmacy management system of the client's chosen pharmacy, removing transcription and handwriting error. You still read what you wrote before sending.

#237 · Management

Before choosing a drug, what question comes first, and which Canadian drug information sources should you use?

Answer

Ask whether a drug is the right tool. Non pharmacological management is frequently first line and frequently the correct CNPLE answer: cognitive behavioural therapy for insomnia, structured exercise for knee osteoarthritis, sodium reduction for stage 1 hypertension in a low risk adult. Use the Compendium of Pharmaceuticals and Specialties, RxTx, Lexicomp and the Canadian Drug Product Database.

#238 · Management

How should a recorded penicillin allergy be handled, and why does it matter?

Answer

It deserves scrutiny rather than automatic avoidance, because most reported penicillin allergies are not true immunoglobulin E mediated allergy. Reflexive substitution drives use of broader spectrum agents, so reassessing the reported reaction is both better care for the client and better stewardship.

#239 · Management

After choosing a candidate drug, what must you screen the choice against?

Answer

Concurrent therapies for interactions, the medication history for what has already failed or caused a reaction, then organ function, pregnancy status, age, ability to use the dosage form, and cost. Provincial formularies differ, and a prescription the person cannot afford is one they will not take.

#240 · Management

What is a Best Possible Medication History, and how does it differ from the chart list?

Answer

A defined Canadian process promoted by Healthcare Excellence Canada and ISMP Canada. A BPMH is not the list in the chart: it is a systematic history built from at least two independent sources and then verified with the client. Each single source is incomplete on its own.

#241 · Management

Name the sources used to build a BPMH, and the limitation of the dispensing record.

Answer

A structured interview with the client or caregiver, inspection of the actual vials and blister packs, the community pharmacy dispensing record, the provincial drug information system where one exists, and the previous discharge summary. The dispensing record shows what was filled, not what was swallowed.

#242 · Management

What product categories must a complete BPMH capture, and which dose is recorded?

Answer

Prescription drugs, over the counter products, vitamins, herbal and natural health products, cannabis, inhalers, eye drops, topicals, injectables, samples, and anything borrowed from family. Record the dose and frequency actually taken rather than the prescribed one, plus the time of the last dose.

#243 · Management

Define medication reconciliation and state how each discrepancy must be resolved.

Answer

The formal comparison of the BPMH against the medications ordered at a transition point. Every discrepancy is resolved as intentional or corrected as an error. Reconciliation is a documented comparison, not a glance at two lists.

#244 · Management

What are the three commonest unintentional medication discrepancies, and where do they occur?

Answer

Omission of a chronic drug, duplication of one drug under both a brand and a generic name, and a dose changed in hospital that was never reconciled at home. Admission, transfer and discharge are the transition points where this fails.

#245 · Management

State the eGFR thresholds that govern metformin use in mL/min/1.73m².

Answer

Metformin should not be started below an eGFR of 45 mL/min/1.73m², needs dose reduction and closer monitoring between 30 and 45, and is contraindicated below 30. Kidney function drives more dose adjustment than any other single factor in prescribing.

#246 · Management

Which renal measure doses direct oral anticoagulants, and how are they monitored?

Answer

Creatinine clearance from the Cockcroft and Gault equation using actual body weight, which is not the same number as the reported eGFR. Dabigatran is the most renally cleared of the group. DOACs need no routine INR but need renal function checked at least annually, more often when it falls, with dose reduction criteria applied exactly.

#247 · Management

Which commonly prescribed drugs accumulate in renal impairment and mimic the underlying disease?

Answer

Gabapentin, pregabalin, allopurinol and most drugs cleared unchanged in the urine. Their toxicity is easily mistaken for progression of the condition being treated, so in a client with declining kidney function the accumulating drug is a leading hypothesis for new confusion or sedation.

#248 · Management

How does cirrhosis change prescribing, and is acetaminophen or an NSAID safer?

Answer

Cirrhosis reduces first pass metabolism and albumin synthesis and impairs clotting factor production, so drugs with high first pass extraction reach far higher concentrations and sedating drugs precipitate encephalopathy. Acetaminophen stays usable in stable liver disease at reduced daily doses and is safer than nonsteroidal anti inflammatory drugs, which precipitate variceal bleeding and hepatorenal syndrome.

#249 · Management

Which three drugs are affected by low albumin, and why can a therapeutic level mislead?

Answer

Phenytoin, warfarin and valproate, all highly protein bound. When albumin is low, the total measured concentration can look therapeutic while the free, active fraction is toxic. The clinical picture outranks the reported level in that situation.

#250 · Management

Name the primary care CYP3A4 inhibitors and inducers.

Answer

Inhibitors: clarithromycin, itraconazole, ketoconazole, ritonavir containing antivirals, and grapefruit juice. Inducers: rifampin, carbamazepine, phenytoin, and St John's wort. Inhibitors raise substrate concentrations toward toxicity, while inducers lower them toward treatment failure.

#251 • Management

What happens when clarithromycin is added to simvastatin, and which statin avoids the problem?

Answer

CYP3A4 inhibition raises simvastatin concentrations and risks rhabdomyolysis. Rosuvastatin is not a CYP3A4 substrate and is the cleaner choice when a client needs a macrolide alongside a statin. Checking the substrate before adding an antibiotic prevents the interaction entirely.

#252 • Management

Why can fluoxetine, paroxetine or bupropion make codeine and tramadol useless?

Answer

CYP2D6 converts codeine and tramadol into their active metabolites, and those three drugs inhibit it, so the client may get no analgesia at all. The mirror risk is an ultrarapid metabolizer, who converts too much and receives a dangerous opioid load from a standard dose.

#253 · Management

Which proton pump inhibitor is preferred with clopidogrel, and why?

Answer

Pantoprazole. CYP2C19 activates clopidogrel and is inhibited by omeprazole and esomeprazole, which blunts the antiplatelet effect. Separately, CYP2C9 clears the active enantiomer of warfarin, which is why variants and inhibitors of that enzyme change INR control.

#254 · Management

How are pediatric doses written, and what cap always applies?

Answer

As milligrams per kilogram per dose or per day, calculated on a weight measured that day, and never exceeding the usual adult dose. Show the calculation in the chart so a pharmacist can check it, and prescribe liquids by milligrams with the matching volume rather than by teaspoons.

#255 · Management

Why are codeine and tramadol avoided in children?

Answer

CYP2D6 conversion to the active metabolite is unpredictable in children, so the same dose can produce no analgesia in one child and a dangerous opioid load in another. Choose an analgesic whose effect does not depend on that conversion step.

#256 · Management

Is avoidance the default when prescribing in pregnancy?

Answer

No. Untreated maternal asthma, epilepsy, depression and hypothyroidism carry real fetal risk, so withholding treatment is itself a harm. What changes in pregnancy is agent selection, not whether the condition is treated.

#257 · Management

Which drugs are avoided in pregnancy, and which source guides lactation decisions?

Answer

ACE inhibitors and angiotensin receptor blockers are avoided because of fetal renal damage and oligohydramnios. Isotretinoin, methotrexate, warfarin and valproate are recognized teratogens. Folic acid starts before conception where pregnancy is planned. For lactation, check a source such as LactMed rather than the product monograph, which advises avoidance for legal rather than clinical reasons.

#258 · Management

How does ageing change drug handling, and why can a creatinine of 90 micromol/L mislead?

Answer

Lean mass and total body water fall while fat mass rises, renally excreted drugs clear more slowly, and sensitivity to central nervous system effects rises. In a frail older adult, low muscle mass keeps creatinine production low, so 90 micromol/L can coexist with badly impaired clearance. Start low and titrate slowly, but do not stop at a subtherapeutic dose.

#259 · Management

What is anticholinergic burden, which ordinary drugs contribute, and what does it cause?

Answer

The cumulative anticholinergic effect across a medication list, driven by oxybutynin, older antihistamines, tricyclic antidepressants and some antipsychotics. It causes confusion, urinary retention, constipation, dry mouth and falls. No single drug looks dangerous alone, which is why the total burden must be counted.

#260 · Management

What is a prescribing cascade, and what is the first hypothesis for a new symptom in an older adult?

Answer

A drug adverse effect misread as a new disease and treated with a second drug: metoclopramide induced parkinsonism treated with levodopa, amlodipine induced ankle edema treated with furosemide. When a new symptom appears in an older adult, the first hypothesis is the medication list.

#261 • Management

How is polypharmacy defined, and what has CIHI reported about older Canadians?

Answer

Commonly five or more concurrent medications, though appropriateness matters more than the count. CIHI has reported that roughly one in four Canadians aged 65 and older is dispensed drugs from ten or more classes in a year. Each added drug raises the risk of interaction, adverse event, hospitalization and non adherence.

#262 • Management

What do the Beers Criteria cover, and name the classes they flag.

Answer

Potentially inappropriate medications in adults 65 and older, from the American Geriatrics Society. Flagged classes include benzodiazepines, non benzodiazepine hypnotics, first generation antihistamines, glyburide, long term nonsteroidal anti inflammatory drugs, and proton pump inhibitors continued beyond eight weeks without a maintained indication.

#263 · Management

How does STOPP and START differ from Beers, and what does the START half do?

Answer

STOPP and START was developed in Ireland, is now in a 2023 third version of roughly 190 criteria, and is organized by body system. Its START half flags evidence based drugs wrongly missing, such as a statin after myocardial infarction or an anticoagulant in atrial fibrillation. Deprescribing is not only subtraction.

#264 · Management

Who does the deprescribing.org proton pump inhibitor algorithm target, and who continues therapy?

Answer

People who finished treatment of uncomplicated reflux or an ulcer and stayed on therapy without an indication. Options are a reduced dose, on demand use, or stopping, warning that rebound acid hypersecretion can occur and is self limited. People with Barrett esophagus, severe esophagitis, or an ongoing bleeding risk continue.

#265 · Management

How fast does the benzodiazepine receptor agonist deprescribing algorithm taper, and what accompanies it?

Answer

Roughly one quarter of the dose every one to two weeks, with slower reductions near the end, paired with cognitive behavioural therapy. It covers benzodiazepines and the z drugs used for insomnia. Abrupt cessation risks seizure and severe withdrawal.

#266 · Management

Which antipsychotic prescriptions does the deprescribing algorithm apply to, and when is a taper tried?

Answer

Those prescribed for behavioural and psychological symptoms of dementia or for insomnia, not primary psychotic illness. Once symptoms have been stable for roughly three months, a taper is attempted with monitoring for recurrence.

#267 · Management

What does the antihyperglycemic deprescribing algorithm target, and which agents are most often reduced?

Answer

Hypoglycemia risk rather than glucose control alone. In frail older adults, glycated hemoglobin targets are relaxed, and glyburide, sulfonylureas generally, and insulin are the agents most likely to need reduction or replacement.

#268 · Management

When does the cholinesterase inhibitor and memantine algorithm support a trial of withdrawal?

Answer

When dementia has advanced, when goals of care shift to comfort, or when no benefit is observable. The algorithm asks whether the drug still delivers benefit rather than whether the diagnosis still exists.

#269 · Management

What four principles make a deprescribing conversation succeed?

Answer

Frame it as optimizing therapy rather than withdrawing care, change one drug at a time, taper wherever withdrawal is possible, and book explicit follow up. The conversation decides success more than the choice of which drug goes first.

#270 · Management

What does the Controlled Drugs and Substances Act govern, and what did the New Classes of Practitioners Regulations change?

Answer

The CDSA is federal law applying identically across Canada, with schedules covering narcotics, controlled drugs, and targeted substances including benzodiazepines. The New Classes of Practitioners Regulations extended federal authority to prescribe controlled substances to nurse practitioners, but each province and territory decides whether and how its NPs use it and what extra education or registration is required.

#271 · Management

What extra requirements apply to controlled substance prescriptions?

Answer

They must be written and signed, narcotics carry no repeats and use part fills instead, and several jurisdictions require a duplicate or secure prescription pad or its electronic equivalent. These requirements sit on top of the standard elements of a valid prescription.

#272 · Management

State the opioid dose thresholds from the 2017 Canadian guideline for chronic non cancer pain.

Answer

Restrict the dose below 90 morphine milligram equivalents per day and preferably below 50, with a trial of tapering recommended above those thresholds. Non opioid and non pharmacological therapy are optimized first, opioids are not first line, and substance use disorder or active psychiatric illness warrants stabilization first.

#273 · Management

At what rate are opioids tapered, and what is the harm of a forced rapid taper?

Answer

Typically 5 to 10 percent of the morphine equivalent dose every two to four weeks, slower at low doses, paused or reversed if the person destabilizes. Forced rapid tapering causes harm, including transition to the unregulated supply, so the taper is negotiated rather than imposed.

#274 · Management

Name the four safeguards that belong in every opioid prescription.

Answer

Check the provincial prescription monitoring program, avoid concurrent benzodiazepines, offer take home naloxone, and document the functional goal the opioid must achieve so continuation can be judged against something concrete rather than against a pain score alone.

#275 · Management

State the core rules of antimicrobial stewardship that generate most exam answers.

Answer

Confirm the infection is bacterial before treating it, since most acute respiratory infections are viral. Do not treat asymptomatic bacteriuria, with pregnancy and pre urologic procedure as the recognized exceptions. Choose the narrowest agent covering the likely organism, guided by local antibiogram data rather than habit.

#276 · Management

How long should antibiotic courses be, and what Canadian campaign frames this?

Answer

The shortest effective duration, since evidence now supports shorter courses for uncomplicated cystitis, community acquired pneumonia and cellulitis. Take cultures where results will change management and de escalate when they return. Choosing Wisely Canada's Using Antibiotics Wisely campaign is the Canadian framing, and delayed prescriptions are one of its tools.

#277 · Management

Beyond antibiotics, which drug classes does stewardship logic extend to?

Answer

Opioids, benzodiazepines, gabapentinoids, proton pump inhibitors and antipsychotics. In each of these, population level overprescribing causes population level harm, so every prescription is a decision with consequences beyond the person in front of you.

#278 · Management

Name the five high alert medication classes where an error produces severe harm.

Answer

Anticoagulants, insulin, opioids, methotrexate and lithium. These are not the drugs most often prescribed wrongly, they are the drugs where a single error produces severe harm, so each one needs an explicit monitoring plan written at the moment it is started.

#279 · Management

What is the usual warfarin INR target, and which drugs raise the INR enough to cause admissions?

Answer

Usually 2.0 to 3.0, with a higher range for some mechanical valves. The interactions that cause admissions are trimethoprim with sulfamethoxazole, metronidazole, fluconazole, macrolides and amiodarone, all of which raise the INR. Anticipate the rise when any of these is started rather than discovering it on the next routine test.

#280 · Management

How is methotrexate dosed for inflammatory disease, and what three safeguards apply?

Answer

Once weekly. Daily dosing is a fatal error and has happened in Canada. Write the day of the week on the prescription, co prescribe folic acid, and avoid trimethoprim containing antibiotics because of additive bone marrow suppression.

#281 • Management

State the lithium maintenance range in mmol/L and the factors that push levels up.

Answer

Maintenance concentrations generally between 0.6 and 1.0 mmol/L, with toxicity risk rising above 1.5 mmol/L. Concentrations climb when sodium falls or the kidney is stressed, so thiazides, ACE inhibitors, angiotensin receptor blockers, nonsteroidal anti inflammatory drugs, dehydration and vomiting are all precipitants.

#282 • Management

Which combination drives hyperkalemia, and what potassium value demands action?

Answer

An ACE inhibitor or angiotensin receptor blocker plus a potassium sparing diuretic such as spironolactone, often with a potassium supplement or a salt substitute on top. Canadian labs report potassium in mmol/L, and a value above 5.5 mmol/L demands action. This is the interaction most often missed.

#283 • Management

Name the three drugs of the triple whammy and explain how they cause acute kidney injury.

Answer

A renin angiotensin blocker, a diuretic, and a nonsteroidal anti inflammatory drug. The NSAID blocks prostaglandin mediated dilation of the afferent arteriole, the renin angiotensin blocker prevents efferent arteriolar constriction, and the diuretic cuts circulating volume. Inflow falls, outflow pressure cannot be held, and filtration collapses. The classic case is an older adult on the first two who buys ibuprofen.

#284 • Management

What four things must a monitoring plan name, and when is it written?

Answer

The parameter you will follow, the target, the interval, and the action threshold that changes management. Write it at the moment you start the intervention, including non pharmacological ones. For a new ACE inhibitor, effectiveness is blood pressure and safety is serum creatinine and potassium at roughly one to two weeks.

#285 · Management

What does CNPLE level medication counselling cover, and why is non compliant a poor label?

Answer

Indication, benefit, cost and coverage, common and rare serious adverse effects with what to do, interactions including over the counter and natural health products, contraindications, how and when to take it, missed dose advice, monitoring, and follow up. Check understanding by teach back. Adherence fails because of cost, complexity, side effects, poor understanding and unasked beliefs, each fixable.

#286 · Counselling

What are the four elements of the motivational interviewing spirit?

Answer

Partnership, working alongside the person rather than delivering expertise downward. Acceptance, honouring their worth and their right to decide. Compassion, meaning the plan serves their welfare and not your metrics. Evocation, drawing the reasons for change out of them, since self generated arguments are the ones people act on.

#287 · Counselling

What does OARS stand for, and which of the four skills is the engine?

Answer

Open questions, Affirmations, Reflective listening and Summaries. Reflective listening is the engine: a simple reflection restates, a complex reflection adds meaning. Affirmations name a specific real strength rather than saying good job. Summaries link the ambivalence together and hand it back to the person.

#288 · Counselling

Which categories of speech count as change talk?

Answer

Desire, ability, reasons, need, commitment, activation, and steps already taken. Change talk is any speech in the direction of change. Notice it and ask for more of it, because the arguments people generate themselves are the ones they act on.

#289 · Counselling

How should a nurse practitioner respond to sustain talk and to discord?

Answer

Reflect sustain talk without amplifying it, and decline the argument. Sustain talk argues for staying the same and is normal in an ambivalent person. If you take the pro change side, the person predictably takes the other side and talks themselves out of changing.

#290 · Counselling

What is the righting reflex, and what does it look like on the exam?

Answer

The urge to fix, warn and correct. It shows up as the option containing accurate information delivered to someone who did not ask for it. Ask permission first: would it be alright if I shared what I am seeing in your bloodwork?

#291 · Counselling

Name the stages of the transtheoretical model, and say how return to use is treated.

Answer

Precontemplation, contemplation, preparation, action and maintenance. Return to use is a normal event in the cycle rather than a failure, so the response is re engagement and problem solving, not discharge from care and not a lecture about wasted effort.

#292 · Counselling

Match each stage of change to the intervention that fits it.

Answer

Precontemplation: raise awareness gently and ask permission. Contemplation: explore ambivalence and build discrepancy with values. Preparation: build a specific plan with dates, doses and supports. Action: provide pharmacotherapy, monitoring and problem solving. Maintenance: identify high risk situations and rehearse responses.

#293 · Counselling

Why does an intervention mismatched to the stage of change cause harm?

Answer

Handing a treatment referral to someone in precontemplation produces discord and often ends the relationship. More awareness raising for someone in preparation wastes the window when they were ready to act. Matching the response to the stage protects both the plan and the relationship.

#294 · Counselling

What do the letters in SBIRT stand for, and when is referral used?

Answer

Screening, Brief Intervention, and Referral to Treatment. Screen the whole practice population with a validated tool, deliver the brief intervention yourself, and reserve referral for risk beyond what primary care can manage alone.

#295 · Counselling

Which validated substance use screening tools are used in Canada, and who gets screened?

Answer

AUDIT-C for alcohol, the ASSIST for multiple substances, and the CRAFFT for adolescents. Screen everyone in the practice population rather than whoever looks like they need it, because selective screening is exactly where implicit bias enters care.

#296 · Counselling

What does a brief intervention actually consist of?

Answer

Five to fifteen minutes of personalized feedback, motivational conversation, and a menu of options the person chooses from. It happens inside a routine visit rather than through a referral, and it ends with something the person selected rather than something you assigned.

#297 · Counselling

List the 5 A's for a short substance use visit, and name the step most often skipped.

Answer

Ask about use, Advise clearly and personally, Assess readiness, Assist with a plan or medication, Arrange follow up. Arrange is the step most often skipped and most often the correct exam answer, because it turns a conversation into care.

#298 · Counselling

Which brief cognitive behavioural strategies fit inside a fifteen minute appointment?

Answer

Behavioural activation schedules a meaningful activity at a set time. A trigger and response plan turns intention into an if then rule. Urge surfing teaches that a craving rises, peaks and falls within twenty to thirty minutes. Cognitive restructuring catches the permission giving thought. Give one, in writing.

#299 · Counselling

What makes a relationship therapeutic, and who holds the power inside it?

Answer

It is purposeful, time limited, and organized around the client's goals rather than yours. The nurse practitioner holds prescribing authority, chart access, and the ability to document a label that follows a person for decades. Naming that imbalance early tends to reduce it.

#300 · Counselling

What are the signs that a therapeutic relationship is working, and the signs it is failing?

Answer

Working: the person raises a concern not disclosed before, corrects you, or arrives with questions. Failing: repeated cancellations, monosyllabic answers where there used to be conversation, a plan agreed to in the room and never enacted, or a persistent sense in you of frustration or rescue.

#301 · Counselling

When a relationship is not therapeutic, what is the professional response, and what counts as abandonment?

Answer

A warm referral. Say plainly that the fit is not serving them, name a colleague or service, transfer information with consent, and keep the door open. Abandonment is failing to arrange the alternative, not the act of transferring care.

#302 · Counselling

Who decides whether care was culturally safe, and who should interpret?

Answer

The person receiving care judges cultural safety, not the provider giving it. Your job is cultural humility: ask, do not assume, accept correction, and offer involvement of family, an Elder or a traditional healer without pressure. Use professional interpreters, since a family interpreter filters content and inherits distress.

#303 · Counselling

What are the four commitments of trauma and violence informed care?

Answer

Understand trauma and violence and their effects on behaviour, create emotionally and physically safe environments, foster choice, collaboration and connection, and take a strengths based capacity building approach. Disclosure is never required. Practise universal precautions: explain before touching, ask permission for each step, and never block the exit.

#304 · Counselling

What is the evidence on Indigenous specific racism in Canadian health care?

Answer

It is documented, not alleged. British Columbia's 2020 In Plain Sight review found widespread stereotyping of Indigenous patients as intoxicated or drug seeking, with missed diagnoses as the consequence. Truth and Reconciliation Commission Calls to Action 18 through 24 address health directly, including cultural safety and anti racism training.

#305 · Counselling

A client becomes tearful, enraged or dissociated mid visit. What do you do?

Answer

Treat the emotion as data. Lower the stimulation, sit rather than stand, and reflect the feeling before the content. For anger keep a calm voice, more than an arm's length of space, and offer a concrete choice. For dissociation, ground: name the room, name the date, feet on the floor.

#306 · Counselling

Define harm reduction, and state what it does not require.

Answer

Harm reduction reduces the health and social harms of substance use without requiring abstinence as a condition of care. Abstinence stays a valid goal for people who want it, but it is never a precondition for receiving help, equipment, medication or follow up.

#307 · Counselling

Which adulterants in Canada's unregulated supply do not respond to naloxone?

Answer

Benzodiazepine adulterants such as etizolam cause prolonged sedation naloxone cannot reverse, so a person may breathe again yet stay deeply unconscious and still need monitoring and airway support. Xylazine, a veterinary sedative, is not an opioid, causes severe skin wounds, and also does not respond to naloxone.

#308 · Counselling

Who should be offered take home naloxone, and what is the response sequence?

Answer

Offer a kit to everyone who uses opioids, to anyone prescribed higher dose opioids, and to their household members. Naloxone was removed from the federal prescription drug list for emergency use outside hospital, so kits need no prescription. Sequence: stimulate, call for help, open the airway and support breathing, give naloxone, repeat if no response.

#309 · Counselling

What protection does the Good Samaritan Drug Overdose Act give someone who calls for help?

Answer

It protects the caller from charges for simple possession of a controlled substance. Teach it explicitly, because fear of arrest is a common reason people delay calling during an overdose, and the delay is what turns a survivable poisoning into a death.

#310 · Counselling

What legal route allows supervised consumption services to operate lawfully in Canada?

Answer

Section 56.1 of the Controlled Drugs and Substances Act lets the Minister of Health grant an exemption, a route created by Bill C-37 in 2017. In *Canada v. PHS Community Services Society*, 2011, the Supreme Court held that refusing Insite an exemption breached section 7 of the Charter.

#311 · Counselling

What do drug checking and needle and syringe programs achieve, and what are their limits?

Answer

Drug checking uses spectrometry or immunoassay strips to tell people what is in a sample, but a negative fentanyl strip never makes a sample safe, because mixing is uneven. Needle and syringe programs reduce HIV and hepatitis C transmission with no evidence of increasing injection rates.

#312 · Counselling

What are managed alcohol programs and prescribed safer supply?

Answer

Managed alcohol programs give measured regular doses of beverage alcohol to people with severe alcohol use disorder and unstable housing, cutting use of non beverage alcohol and emergency service use. Prescribed safer supply offers pharmaceutical alternatives, most often hydromorphone, to people at high risk of death. It remains genuinely contested.

#313 · Counselling

Which opioid agonist therapy does Canadian guidance place first line, and why?

Answer

Buprenorphine with naloxone. Its partial agonist ceiling effect makes fatal respiratory depression far less likely than with a full agonist. Methadone is an effective alternative when buprenorphine is not tolerated or not effective, and slow release oral morphine is a further alternative in some jurisdictions.

#314 · Counselling

What extra monitoring does methadone require compared with buprenorphine with naloxone?

Answer

Closer monitoring overall, including QT interval prolongation and accumulation during the first two weeks, when dose increases carry the most risk. Methadone is a full agonist with no ceiling effect, so respiratory depression risk is higher than with buprenorphine and naloxone.

#315 · Counselling

Compare standard buprenorphine induction with low dose initiation.

Answer

Standard induction waits for objective withdrawal, conventionally a Clinical Opiate Withdrawal Scale score of about 12 or more, because starting too early precipitates withdrawal. Low dose initiation overlaps tiny increasing doses with continued opioid use, which avoids that in the fentanyl era. Confirm current provincial protocols rather than memorizing one schedule.

#316 · Counselling

Is nurse practitioner authority to prescribe opioid agonist therapy the same across Canada?

Answer

No. Authority comes from the New Classes of Practitioners Regulations under the Controlled Drugs and Substances Act, then is shaped by each province or territory and each regulatory college. The federal methadone exemption ended in 2018. Treat any option stating a single national rule as suspicious.

#317 · Counselling

Which medications are first line for alcohol use disorder, and what is the key contraindication?

Answer

Naltrexone and acamprosate. Naltrexone reduces heavy drinking and is contraindicated in anyone taking opioids, including opioid agonist therapy. Acamprosate supports abstinence and is renally cleared. Disulfiram is a later option that needs supervision and full informed consent.

#318 · Counselling

What comes first in alcohol withdrawal management, and why is thiamine timed carefully?

Answer

Assess withdrawal risk first: past withdrawal seizures or delirium tremens, heavy daily intake and significant comorbidity push toward inpatient management. Benzodiazepines treat the withdrawal itself. Give parenteral thiamine before any glucose containing fluid, to avoid precipitating Wernicke encephalopathy.

#319 · Counselling

What does Canada's Guidance on Alcohol and Health say, and what is a standard drink?

Answer

Published in 2023, it replaced the old low risk drinking guidelines with a continuum of risk: 2 standard drinks or fewer per week is low risk, 3 to 6 is moderate, and 7 or more is increasingly high risk. A Canadian standard drink holds 17.05 mL of pure alcohol, such as 341 mL of 5 percent beer.

#320 · Counselling

Give the person first alternatives to the stigmatizing terms used in substance use care.

Answer

Person who uses drugs, not addict. Substance use disorder, not abuse. Positive or negative urine drug test, not clean or dirty. Return to use, not relapse. A baby is born with neonatal opioid withdrawal syndrome, not addicted. Language changes how clinicians actually respond.

#321 · Transition of Care and Documentation

What counts as a transition of care, and what three risks does every transition create?

Answer

Any point where accountability moves: admission, discharge home, referral, transfer between units, a move to long term care, a handoff to an on call colleague, and the end of your own relationship with a client. Information is lost, accountability becomes ambiguous, and the plan fails to change with the setting.

#322 · Transition of Care and Documentation

The blueprint says the nurse practitioner leads transitions. What does leading mean?

Answer

Four things: collaborate with the client on what they need in the new setting, arrange the resources, drug therapy, tests, procedures and follow up that carry the plan forward, transfer the information the receiver needs, and modify the plan to fit where the client is going.

#323 · Transition of Care and Documentation

When does discharge planning begin, and what is assessed besides the diagnosis?

Answer

At the first encounter, because where the person will go shapes decisions made days earlier. Assess who is at home, the stairs, whether they can afford the medication, and transport to follow up. A client without stable housing cannot store insulin, and instructions in an unreadable language were never given.

#324 · Transition of Care and Documentation

When is a transfer of accountability actually complete?

Answer

Not when you have spoken. It is complete when the receiver has the information, has had a chance to ask questions, and has accepted responsibility for the next steps. Until that acceptance happens, the client and the outstanding tasks are still yours.

#325 · Transition of Care and Documentation

What does each letter of SBAR carry in a handover?

Answer

Situation states who the client is and what is happening now. Background gives the relevant history, not the whole history. Assessment states what you believe is going on, your judgment rather than raw numbers alone. Recommendation states what you want done, and by when.

#326 · Transition of Care and Documentation

Which part of SBAR is most often weakened, and what fixes it?

Answer

Recommendation. A handover ending in description leaves the next step unowned. Saying the creatinine is up is description. Saying it rose from 78 to 164, that you held the ACE inhibitor, and that she needs reassessment within twelve hours is a handover. Read back critical numbers and doses.

#327 · Transition of Care and Documentation

What belongs in a safe discharge summary?

Answer

Reason for the encounter, discharge diagnosis, what was done, the medication list noting what was started, stopped, held or changed and why, pending results with owners, the follow up plan with named clinicians and dates, functional and social status, red flags, and your contact information.

#328 · Transition of Care and Documentation

Why are pending results the highest risk item in a discharge summary?

Answer

A test that results after discharge belongs to no one unless the summary names an owner. The correct answer names a person and a timeframe, not the clinic or the team in general, because a result assigned to everybody is followed up by nobody.

#329 · Transition of Care and Documentation

What are the three steps of medication reconciliation, and at which points is it done?

Answer

Build a best possible medication history from at least two sources, compare it against what is ordered in the new setting, then resolve every discrepancy and document the reason for each intentional change. Reconcile at admission, at every internal transfer, and at discharge, not only at discharge.

#330 · Transition of Care and Documentation

Distinguish an intentional from an unintentional medication discrepancy.

Answer

An intentional discrepancy is a clinical decision, and its failure mode is undocumented intent, so a later clinician restarts a drug stopped for a good reason nobody wrote down. An unintentional discrepancy is an error: a drug omitted, duplicated, or carried forward at the wrong dose.

#331 · Transition of Care and Documentation

What should a best possible medication history capture, and who else receives the list?

Answer

What the client actually takes rather than what they are supposed to take, including over the counter products, herbals, cannabis, inhalers and injections given elsewhere. Give the client a written reconciled list in plain language, and send the same list to the pharmacy and the primary care provider.

#332 · Transition of Care and Documentation

What three jobs does the health record do, and what is assumed about undocumented care?

Answer

Clinical communication, legal document, and evidence of your reasoning. Canadian investigations, inquests and civil actions operate on the assumption that care not documented was care not provided. That is not a formal rule of law, but memory years later is weak against a contemporaneous chart showing nothing.

#333 · Transition of Care and Documentation

What is rationale in documentation, and for which decisions does it matter most?

Answer

The reasoning connecting findings to decision. Recording that you prescribed amoxicillin is data; recording that the client met criteria, had no penicillin allergy and declined watchful waiting is reasoning. Rationale matters most for decisions that look wrong from outside: why you did not image, why you accepted a refusal, why you departed from a guideline.

#334 · Transition of Care and Documentation

How should refusals and adherence problems be written in the chart?

Answer

In objective language. Non compliant is a judgment; metformin taken three days per week because of diarrhea is a record. Document what you told the client, what they refused, and what you advised about the consequences of refusing. Telephone and secure message advice gets the same treatment.

#335 · Transition of Care and Documentation

How do you correct an error on paper and in an electronic record?

Answer

On paper, draw a single line through the error so the original stays readable, write the correction beside it, and add your initials, date and time. Electronically, use the amendment function, which keeps an audit trail of the original, the amendment, who changed it and when. Nothing is deleted.

#336 · Transition of Care and Documentation

When is a late entry acceptable, and what turns one into fraud?

Answer

Late entries are acceptable and sometimes unavoidable. Label them as late entries, time them with the actual moment of writing, and state when the events occurred. Backdating an entry to look contemporaneous is fraud, and an unlabelled addition found in an audit trail damages you more than the original gap.

#337 · Transition of Care and Documentation

What does PIPEDA apply to, and what do its principles cover?

Answer

The federal Personal Information Protection and Electronic Documents Act applies to personal information handled in the course of commercial activity and to federally regulated workplaces. Its ten fair information principles cover accountability, consent, limited collection and use, accuracy, safeguards, individual access, and challenging compliance.

#338 · Transition of Care and Documentation

Is Ontario's PHIPA the national health privacy law for Canadian nurse practitioners?

Answer

No. PIPEDA steps back where a province has enacted health privacy legislation declared substantially similar. PHIPA is the most cited example and only an example. Alberta has the Health Information Act, Saskatchewan its own Health Information Protection Act, and Quebec runs its own regime strengthened by Law 25. Thresholds and retention periods differ.

#339 · Transition of Care and Documentation

Who is the health information custodian when a nurse practitioner is employed, and when self employed?

Answer

The custodian, called a trustee or steward in some provinces, holds lawful custody or control of the information. A nurse practitioner employed by a hospital is usually an agent of the hospital, which is the custodian. A self employed nurse practitioner is typically a custodian personally.

#340 · Transition of Care and Documentation

Explain the circle of care, and what a lockbox does to sharing inside it.

Answer

Within the circle of care, the clinicians and staff directly involved in that client's care, consent to share for that purpose is generally implied. Outside it, express consent is required. Where a lockbox applies you may not share the withheld information, and you must tell the receiver something was withheld without revealing what.

#341 · Transition of Care and Documentation

Which security measures are expected, and what is the most common privacy offence?

Answer

Unique credentials with no shared logins, encryption of devices and data in transit, role based access, screen locks, secure paper disposal, audit logging, and training for everyone who touches the record. Snooping, meaning access without a care purpose, is the most common privacy offence by Canadian health professionals.

#342 · Transition of Care and Documentation

What are the four steps of privacy breach response, and what does notification include?

Answer

Contain, notify, investigate, prevent recurrence. Containment comes before paperwork: retrieve the misdirected fax, wipe the lost device, suspend the account. Tell the affected person what happened, what was involved, what you have done, and how to complain to the privacy commissioner. Concealing a breach is worse than the breach.

#343 · Transition of Care and Documentation

How long are health records kept, and what counts as secure destruction?

Answer

Retention comes from legislation, college standards and employer policy, and differs across the country. A common benchmark is ten years from the last entry for an adult and ten years past the age of majority for a child, but your jurisdiction governs. Destroy by cross cut shredding or certified wiping, and log it.

#344 · Transition of Care and Documentation

Who owns the chart, and what can a client actually demand?

Answer

Under the Supreme Court decision in *McInerney v. MacDonald*, the physical record belongs to the clinician or facility that created it, while the information belongs in a practical sense to the client, who may access it and control its disclosure. A client can demand a copy, not the original chart.

#345 · Transition of Care and Documentation

What must a self employed nurse practitioner do when the practice closes?

Answer

Notify clients, let them obtain a copy or transfer the record, and name a successor custodian for the remaining retention period. Self employed practice also carries personal responsibility for liability protection, a privacy program, and coverage during absences. The Canadian Nurses Protective Society is the route to legal advice.

#346 · Transition of Care and Documentation

How are nurse practitioners compensated in Canada, and does the model change their obligations?

Answer

Salaried employment dominates, with contract or sessional blocks paying for defined service periods, and fee for service or panel based public billing available in some jurisdictions only. Business independence is not independence from regulation: the same standards, scope, documentation duties and duty to practise within competence apply.

#347 · Transition of Care and Documentation

What makes billing and advertising ethical in an independent nurse practitioner practice?

Answer

Bill only for services actually provided, by the person who provided them, at the code matching what was done. Upcoding, charging privately for publicly insured services, and tying access to uninsured extras carry regulatory and criminal consequences. Advertising must be truthful and verifiable, with no guaranteed outcomes, and testimonials are widely restricted.

#348 · Transition of Care and Documentation

What must a client be told before a virtual visit for consent to be informed?

Answer

The benefits, the limitations, specifically that you cannot examine them and a diagnosis may be less certain, the platform's privacy and security risks, the in person alternative, and what happens if the technology fails. Consent to virtual care is separate from consent to treatment, may be verbal, and belongs in the chart.

#349 · Transition of Care and Documentation

When is virtual care not appropriate, and what are its two named failure modes?

Answer

When a physical finding would change management, when the presentation could plausibly be an emergency, when vital signs are needed and cannot be obtained, for most acute abdominal, chest and neurological complaints, when technology or privacy is lacking, or when the client asks to be seen. The failure modes are delay and fragmentation.

#350 · Transition of Care and Documentation

How do assessment and documentation change in virtual care?

Answer

Take more history, not less, and use the client as your hands: ask them to point, press, count respirations and read out home readings. Confirm identity and location, and ask who else is present. Document modality, both locations, identity verification, consent, technical limits and safety netting. You must be licensed where the client is.

#351 · Quality Improvement and Research

What is a practice gap, and what makes one actionable?

Answer

The distance between what care should look like and what your data says it actually looks like. A suspected gap is a hypothesis. A measured gap, such as an audit showing only 48% of your patients with type 2 diabetes had an albumin to creatinine ratio last year, is a starting point a team can act on.

#352 · Quality Improvement and Research

How do you tell an evidence gap from an implementation gap?

Answer

If nobody knows the right answer, that is a research question. If the right answer is well established but your practice does not deliver it reliably, that is a quality improvement problem. Searching the literature on hand hygiene will not raise your clinic's hand hygiene rate.

#353 · Quality Improvement and Research

What makes a randomized controlled trial the strongest design for a treatment question?

Answer

Randomization is the only method that distributes unknown confounders as well as known ones, which is what supports a causal claim. It protects nothing if allocation was not concealed or if many participants dropped out, so appraise concealment and attrition before accepting the result.

#354 · Quality Improvement and Research

What does a cohort study give you, and where is it weak?

Answer

It follows a group forward and compares exposed with unexposed people, yielding incidence and relative risk directly, and it suits questions where randomization would be unethical. Its weakness is confounding, since people who choose an exposure differ systematically from those who do not.

#355 · Quality Improvement and Research

When is a case control design the right choice, and what measure does it produce?

Answer

For rare outcomes and long latency periods, because it starts with the outcome and looks backward for exposure. It cannot give incidence, so it produces odds ratios rather than risk ratios, and it is vulnerable to recall bias and to how controls were selected.

#356 · Quality Improvement and Research

What can and cannot be concluded from a cross sectional study?

Answer

It measures exposure and outcome at one moment, so it yields prevalence, and with an independent reference standard it is the top design for diagnostic accuracy. It cannot establish which came first, so causal language in a cross sectional paper is a red flag.

#357 · Quality Improvement and Research

How is a qualitative study judged?

Answer

On credibility, transferability, dependability and confirmability, not on sample size. Designs include phenomenology, grounded theory, ethnography and interpretive description, and they answer how people experience illness and why a program failed. Ranking a qualitative study as weak evidence for a qualitative question is a category error.

#358 · Quality Improvement and Research

Is a systematic review always stronger evidence than a single good trial?

Answer

No. A systematic review applies a prespecified protocol to find, appraise and synthesize eligible studies, and a meta analysis pools them statistically, but the pooled result inherits every weakness of the studies inside it. A poor meta analysis of six biased trials sits below one large well run cohort.

#359 · Quality Improvement and Research

What does PICO structure, and what does adding T do?

Answer

Population, Intervention, Comparison and Outcome, with Timeframe added when it matters. Does exercise help depression is unsearchable. Naming the population, supervised aerobic exercise, usual primary care, symptom scores and 12 weeks tells you which studies count and which do not.

#360 · Quality Improvement and Research

Distinguish internal validity from external validity.

Answer

Internal validity asks whether the study got the right answer for the people in it. External validity asks whether that answer transfers to your patient. A trial can be internally flawless and useless to you when it enrolled younger single disease participants and your patient is 82 with six conditions.

#361 · Quality Improvement and Research

What is selection bias, and what defends against it?

Answer

Systematic error arising when the compared groups differ at baseline, or when who gets studied depends on the outcome itself. It concerns who entered the study rather than how they were measured, and randomization with concealed allocation is the main defence.

#362 · Quality Improvement and Research

What is measurement bias, and for which outcomes does blinding matter most?

Answer

Measurement bias occurs when outcomes are assessed differently between the groups. Blinding of participants, clinicians and outcome assessors matters most for subjective outcomes such as pain, where expectation shapes both reporting and rating. Hard outcomes such as death are less vulnerable.

#363 · Quality Improvement and Research

At what dropout rate should a trial raise suspicion, and why?

Answer

Attrition bias becomes a real concern beyond roughly 20% loss to follow up. The number itself is not the problem; the problem is whether dropouts differ between the groups, because differential loss can manufacture an apparent effect or erase a real one.

#364 · Quality Improvement and Research

What is confounding, and how is it addressed?

Answer

A third variable related to both the exposure and the outcome. It is addressed by randomization, restriction, matching, stratification or multivariable adjustment. Residual confounding always remains in observational work, which is why an observational association is never proof of causation.

#365 · Quality Improvement and Research

Compare intention to treat with per protocol analysis.

Answer

Intention to treat analyses participants in the group they were randomized to, whatever they actually took, preserving randomization and giving the more honest estimate of real world effectiveness. Per protocol analyses only those who completed treatment as assigned, estimating efficacy under ideal adherence while breaking randomization. If only per protocol is reported, ask why.

#366 · Quality Improvement and Research

Define a p value, and name three things it is not.

Answer

The probability of a result at least as extreme as the one observed, assuming the null hypothesis is true. It is not the probability that the null hypothesis is true, not the probability the finding is a fluke, and not a measure of effect size.

#367 · Quality Improvement and Research

Which null value does a confidence interval straddle for ratio measures, and for difference measures?

Answer

For ratio measures such as risk ratio, odds ratio and hazard ratio the null value is 1, so an interval including 1 is not statistically significant. For difference measures such as a mean difference the null value is 0. The width of the interval tells you the precision.

#368 · Quality Improvement and Research

Controls have a 20% event rate and treated patients 15%. Give the absolute risk reduction and NNT.

Answer

The absolute risk reduction is 20% minus 15%, which is 5 percentage points, or 0.05. The number needed to treat is $1 \div 0.05 = 20$, so 20 patients must be treated to prevent one event.

#369 · Quality Improvement and Research

In that same trial, 20% versus 15%, what are the risk ratio and the relative risk reduction?

Answer

The risk ratio is $0.15 \div 0.20 = 0.75$, and the relative risk reduction is 1 minus 0.75, which is 25%. Relative measures stay stable across populations, so on their own they say nothing about how much benefit a given patient can expect.

#370 · Quality Improvement and Research

Control event rate 0.4%, treatment 0.3%, the same 25% relative reduction. What is the NNT?

Answer

Since $0.003 \div 0.004 = 0.75$, the relative risk reduction is still 25%. The absolute risk reduction is 0.1 percentage points, or 0.001, so the number needed to treat is $1 \div 0.001 = 1000$. Both trials could advertise a 25% reduction while one treats 20 patients per event prevented and the other 1000.

#371 · Quality Improvement and Research

Serious bleeding occurs in 3% of treated patients and 1% of controls. Give the number needed to harm.

Answer

The absolute risk increase is 3% minus 1%, which is 0.02, so the number needed to harm is $1 \div 0.02 = 50$. Set beside a number needed to treat of 20, preventing one event for every 20 treated while harming one in 50 is a defensible trade off.

#372 · Quality Improvement and Research

Distinguish risk from odds, then compute both for 40 of 200 treated and 60 of 200 controls.

Answer

Risk is events $\div$ everyone in the group, and odds is events $\div$ non events. Risk is $40 \div 200 = 0.20$ and $60 \div 200 = 0.30$, so the risk ratio is 0.67. Odds is $40 \div 160 = 0.25$ and $60 \div 140 = 0.43$, so the odds ratio is 0.58.

#373 · Quality Improvement and Research

Why should an odds ratio of 0.58 not be described as 42% less likely?

Answer

An odds ratio overstates the effect whenever the outcome is common, converging with the risk ratio only when the outcome is rare, roughly under 10%. In that same trial the risk ratio was 0.67, not 0.58. Case control studies and logistic regression both report odds ratios.

#374 · Quality Improvement and Research

Which six domains does AGREE II appraise, and which two weigh most for a clinician?

Answer

Scope and purpose, stakeholder involvement, rigour of development, clarity of presentation, applicability, and editorial independence. Rigour of development and editorial independence carry the most weight for a clinician deciding whether to trust the document.

#375 · Quality Improvement and Research

Two credible Canadian guidelines disagree. What do you do?

Answer

Appraise both with AGREE II, isolate the exact point of divergence, examine the primary evidence behind it, then decide with the patient. Panels may have searched different literature, used different cutoff dates, or valued outcomes differently. A documented defensible deviation is safer than silent compliance with a guideline that does not fit.

#376 · Quality Improvement and Research

How does CIHR define knowledge translation, and what separates integrated from end of grant forms?

Answer

A dynamic and iterative process of synthesis, dissemination, exchange and ethically sound application of knowledge to improve health and strengthen the health system. In integrated knowledge translation the knowledge users shape the question from the start. End of grant knowledge translation happens after a study closes.

#377 · Quality Improvement and Research

What are the parts of the knowledge to action cycle?

Answer

Knowledge creation sits at the centre as a funnel: inquiry, synthesis, then tools such as guidelines and decision aids. Around it runs the action cycle: identify the problem and select knowledge, adapt it locally, assess barriers, tailor interventions, monitor use, evaluate outcomes, and sustain use. Local adaptation usually decides success.

#378 · Quality Improvement and Research

At which levels do implementation barriers cluster, and what follows from knowing the level?

Answer

Individual, including unawareness, disagreement and inertia. Team, including unclear roles, weak leadership and absent feedback. System, including time, cost and an electronic record that makes the right action harder than the wrong one. Match the intervention to the barrier: education fixes an awareness problem and does nothing for a workflow problem.

#379 · Quality Improvement and Research

How does quality improvement differ from research?

Answer

Research asks what works, produces generalizable knowledge, and follows a fixed protocol. Quality improvement asks whether a known good practice is being delivered here, using rapid sequential changes in one local setting. Intent at the outset determines which pathway a project is on.

#380 · Quality Improvement and Research

Does a quality improvement project need research ethics board review?

Answer

Under TCPS 2 Article 2.5, quality assurance and improvement studies and program evaluation used exclusively for assessment, management or improvement purposes do not require review. The moment the intent shifts to generalizable knowledge, for example randomizing patients to an unproven intervention, review is required. Consult the board when unclear.

#381 · Quality Improvement and Research

What three questions does the Model for Improvement ask before any change?

Answer

What are we trying to accomplish? How will we know that a change is an improvement? What change can we make that will result in improvement? The first produces a SMART aim, the second a measurement plan, and the third the changes you will test.

#382 · Quality Improvement and Research

What belongs in a measurement plan, and what does a SMART aim look like?

Answer

Outcome measures, the result for patients. Process measures, whether the steps actually happen. Balancing measures, what might worsen elsewhere, such as visit length. A SMART aim names the number and the date: raise documented albumin to creatinine ratio testing from 48% to 80% by March 31.

#383 · Quality Improvement and Research

Walk through the four steps of a single PDSA cycle.

Answer

Plan the test and predict the result. Do it on a tiny scale, one clinician and five patients on one afternoon. Study the result against the prediction. Act by adopting, adapting or abandoning, then run the next cycle. Small fast cycles beat one large launch.

#384 · Quality Improvement and Research

What counts as a signal of non random change on a run chart?

Answer

A shift, meaning six or more consecutive points on one side of the median centre line. A trend, meaning five or more consecutive points all rising or all falling. And an astronomical point far outside the usual pattern. Measurement over time beats a before and after comparison.

#385 · Quality Improvement and Research

What does a control chart add to a run chart?

Answer

Limits, usually at 3 standard deviations from the mean, which separate common cause variation, the normal noise of the process, from special cause variation, which has an assignable reason. Reacting to common cause variation as though it were special usually makes performance worse.

#386 · Quality Improvement and Research

Contrast root cause analysis with failure modes and effects analysis.

Answer

Root cause analysis is retrospective: after a harmful incident the team reconstructs the timeline, asks why repeatedly, maps contributing factors on a fishbone diagram, and recommends system level change. Blaming an individual marks a weak analysis. Failure modes and effects analysis is prospective, scoring severity, occurrence and detectability before launch.

#387 · Quality Improvement and Research

How does just culture respond to human error, at risk behaviour and reckless behaviour?

Answer

Human error, an inadvertent slip, calls for consoling the individual and fixing the system that allowed it. At risk behaviour, a drift into a shortcut whose risk is no longer recognized, calls for coaching. Reckless behaviour, conscious disregard of a substantial unjustifiable risk, calls for discipline. Judge the behaviour, not the outcome.

#388 · Quality Improvement and Research

What does disclosure of harm require, and what do apology laws protect?

Answer

Initial disclosure as soon as reasonably possible once the patient is stable, covering known facts, clinical consequences, the care plan and an expression of regret, without speculating about cause. Post analysis disclosure follows the review and shares what will change. Most provinces have apology legislation protecting sympathy from being treated as an admission of liability.

#389 · Quality Improvement and Research

What are the three core principles of TCPS 2, and what makes research consent valid?

Answer

Respect for persons grounds free and informed consent. Concern for welfare weighs physical, mental, spiritual, economic and social wellbeing against risk. Justice requires fair treatment and fair distribution of burdens and benefits. Consent is a process, not a signature, and participants may withdraw at any time without penalty to their care.

#390 · Quality Improvement and Research

What do the OCAP principles mean, and to whom do they apply?

Answer

Ownership, control, access and possession: First Nations principles asserted and trademarked by the First Nations Information Governance Centre. A community collectively owns its information, controls all research stages affecting it, can reach information about itself, and physically stewards its data. OCAP is specifically First Nations; Inuit and Métis assert their own approaches.

#391 · Leader

How does the CNPLE blueprint define leadership, and what three working parts does it name?

Answer

Collaborative leadership within the health care system, built from striving for a culture of excellence, facilitating effective teams, and supporting communication within complex health systems. The definition does not say directing, supervising or managing. The model leader makes the surrounding system work better.

#392 · Leader

What does the blueprint term Leadership in Place mean?

Answer

Leadership does not flow only from a hierarchical position. Leadership roles are available to everyone, at every level, regardless of title. A new graduate nurse practitioner with no formal authority still carries a leadership obligation when she notices a problem nobody else has named.

#393 · Leader

A stem shows an NP who spots a safety pattern and waits for someone senior to act. Correct?

Answer

No. Under Leadership in Place, waiting for a title holder is almost never the keyed answer, and acting far outside your role without telling anyone is also wrong. The correct response usually sits between: raise it, own a piece of it, bring the right people in.

#394 · Leader

When does transformational leadership fit, and where does it fit poorly?

Answer

It works through vision, inspiration and individual attention, so it fits culture change and sustained improvement such as reducing wait times over a year or rebuilding morale. It is slow and depends on trust, which makes it a poor fit for a crisis.

#395 · Leader

When does transactional leadership fit, and what is its built in weakness?

Answer

It works through clear expectations, monitoring and consequences, fitting routine, high volume, safety critical work where variation is the enemy, such as hand hygiene audits and documentation standards. Its weakness is compliance rather than commitment, and compliance stops when the monitoring stops.

#396 · Leader

Which leadership style best fits a burned out, demoralized team, and what does it look like?

Answer

Servant leadership, which puts the growth and needs of the team first on the theory that a supported team serves patients better. An NP who spends her first month asking each staff member what gets in the way of their work, then removing those obstacles, is practising it.

#397 · Leader

What is situational leadership, and what is the common exam error about it?

Answer

The right approach depends on the follower's competence and commitment for a specific task. A new graduate learning wound care needs direction; the same nurse two years later needs delegation. The common error is applying one style to a whole person rather than to a task.

#398 · Leader

Distinguish collaborative leadership from distributed leadership, and link both to Leadership in Place.

Answer

Collaborative leadership shares decision making across professions and treats expertise as distributed rather than concentrated. Distributed leadership holds that leadership is a property of the team, with different people leading depending on who holds the relevant knowledge. In a code that is whoever has the airway skill. Both express Leadership in Place.

#399 · Leader

Name Tuckman's five stages of team development in order.

Answer

Forming, polite and tentative while members test what is acceptable. Storming, conflict over roles, influence and direction. Norming, settling into shared expectations. Performing, productive problem solving without the leader arbitrating. Adjourning, closing the work and disbanding the team.

#400 · Leader

Why is storming not a sign of team failure, and what happens if a leader suppresses it?

Answer

Storming is normal and necessary. A leader who suppresses it produces a team that looks calm and performs poorly, because the disagreements go underground. Teams also do not move in a straight line: a new member, manager or mandate can push a performing team back.

#401 · Leader

Define psychological safety and state why it matters most for team performance.

Answer

The shared belief that speaking up will not lead to humiliation or punishment. It is the strongest single predictor of whether a team catches its own errors. A team without it produces silent staff, unreported near misses, and a leader who believes everything is fine.

#402 · Leader

What specific leader behaviours build psychological safety?

Answer

Unglamorous ones: admitting your own mistakes, thanking people for raising concerns even when the concern proves unfounded, and meeting bad news with curiosity instead of blame. A team where the least senior member can say wait, I think that is the wrong side, and be thanked, has it.

#403 · Leader

NP scope overlaps with several professions. Is that overlap itself the problem?

Answer

No. Overlap with family physicians, RNs, RPNs, pharmacists, physiotherapists and social workers is not a defect, because shared competencies let teams cover for each other. Overlap becomes conflict only when nobody has said out loud who does what.

#404 · Leader

An NP and a pharmacist both adjust antihypertensive doses and each privately thinks the other oversteps. What is really wrong?

Answer

Neither is out of scope, so the conflict looks interpersonal but is structural: no one agreed who owns which patients. The result is duplicated visits and contradictory advice. The leadership response is to name the ambiguity, convene the two roles, and write down the working agreement, including how disagreements about a patient get settled.

#405 · Leader

Name the six competency domains of the Canadian Interprofessional Health Collaborative framework.

Answer

Interprofessional communication; patient, client, family and community centred care; role clarification; team functioning; collaborative leadership; and interprofessional conflict resolution. The first two support all the others, and conflict resolution is treated as a core competency rather than an unfortunate necessity.

#406 · Leader

A co located team has weekly meetings and a shared record, yet patients still repeat their story. What is missing?

Answer

Not proximity or technology. Co location alone produces parallel practice, where each professional sees the same patient and writes a note nobody reads. What is missing is role clarification and a shared care plan with an identified coordinator.

#407 · Leader

What does genuine interprofessional collaboration require beyond putting people in one building?

Answer

Shared goals, a care plan the patient helped shape, mutual understanding of what each profession contributes, structured communication such as huddles or case conferences, and an agreed process for handling disagreement.

#408 · Leader

In interest based negotiation, what is the difference between a position and an interest?

Answer

A position is what someone says they want; an interest is why they want it. Positions collide, and interests often do not. Making the interests visible reveals options that neither stated position contained.

#409 · Leader

Which four moves shift a conflict from positions to interests?

Answer

Separate the people from the problem. Ask why and why not rather than restating your position. Generate several options before evaluating any. Judge the options against an objective standard such as the guideline or the patient's own goals.

#410 · Leader

Two clinicians disagree about anticoagulation for a frail patient. What settles it, and what does not?

Answer

Seniority does not settle it. The patient is a party to the negotiation, not the object of it, so the resolving question is what this patient values when the tradeoff is stroke risk against bleeding risk.

#411 · Leader

When do you negotiate a clinical disagreement, and when do you escalate instead?

Answer

Negotiate when the disagreement is about judgment within acceptable practice. Escalate when a colleague's practice is unsafe. Knowing which of the two you are looking at is what the exam is testing.

#412 · Leader

What is the correct order for escalating a colleague's unsafe practice?

Answer

Speak to the colleague first, in private, naming the specific behaviour and the specific risk. Then take it to the next level, usually the clinical or medical lead, in writing, with facts and dates rather than characterizations. Document what you observed, what you did and who you told.

#413 · Leader

Which two situations override speaking to the colleague privately first, and what comes before anything else?

Answer

Immediate patient harm, or a person who is unsafe to approach. If the risk is immediate, protect the patient first: stop the procedure, hold the medication, stay with the patient. If internal channels fail on competence or conduct, provincial legislation generally obliges reporting to the college, and one mention to a busy manager does not discharge that duty.

#414 · Leader

List Kotter's eight steps for large scale change.

Answer

Create urgency, build a guiding coalition, form a strategic vision, enlist a volunteer army, enable action by removing barriers, generate short term wins, sustain acceleration, and institute change so it becomes the new normal. The first and last steps are skipped most often.

#415 · Leader

Why do the first and last of Kotter's steps matter most in practice?

Answer

Change launched without urgency is ignored. Change never anchored in routines, orientation and job descriptions quietly reverts within a year. Skipping either end wastes the work done in the middle.

#416 · Leader

Describe Lewin's three stage change model and what each stage means in a clinic.

Answer

Unfreeze means loosening the current way of working by making the case that it no longer serves. Change is the transition itself, uncomfortable and temporarily less efficient. Refreeze means stabilizing the new way as the default.

#417 · Leader

Nurses resist a new screening tool. How should a leader read that resistance?

Answer

As information rather than obstruction. They are often telling you something true: it adds seven minutes to a visit with no seven minutes in it, it duplicates an intake question, or the last three initiatives were abandoned halfway. Ask what would make it workable, and the people who resisted become the people who fix it.

#418 · Leader

Name the five conditions the Canada Health Act sets for full federal transfer payments.

Answer

Public administration, comprehensiveness, universality, portability and accessibility. A provincial or territorial insurance plan must meet all five to receive full federal transfer payments.

#419 · Leader

Who is responsible for delivering health services in Canada, and what is the federal role?

Answer

Delivery and administration are provincial and territorial, so there is no single Canadian system but fourteen or more. The federal role is narrower: the Canada Health Act conditions, plus services for specific populations including First Nations and Inuit communities through Indigenous Services Canada and Non-Insured Health Benefits, Armed Forces members, veterans, federal inmates and some refugee claimants.

#420 · Leader

Which aspects of nurse practitioner practice vary by Canadian jurisdiction?

Answer

Scope of practice, prescribing authority for controlled substances, authority to admit and discharge, and eligibility to be the most responsible provider. Employment models vary too. A question may hinge on whether the NP can act or must first negotiate a medical directive, collaborative agreement or privileging process.

#421 · Leader

How does Jordan's Principle resolve a federal and provincial funding standoff for a First Nations child?

Answer

The government of first contact pays for the needed service and sorts out jurisdiction afterward. It exists because children were caught between a federally funded nursing station, a provincially funded hospital and a medication covered by neither. An NP who knows this can resolve a delay that would otherwise take months.

#422 · Leader

Define surge capacity and name the four elements it is planned across.

Answer

The ability to expand services rapidly beyond normal limits. Staff means calling in, cross training and expanding scope under emergency provisions. Stuff means supplies, medications, oxygen and equipment. Structure means space, including alternate care sites. Systems means the policies, communication and coordination holding the other three together.

#423 · Leader

What is the Incident Command System, and what four sections support the incident commander?

Answer

The standard emergency structure Canadian health organizations use, adapted in many provinces as the Incident Management System. One incident commander holds overall authority, supported by operations, planning, logistics, and finance and administration.

#424 · Leader

What are the strengths of incident command, and what changes for an NP in a declared emergency?

Answer

A single chain of command, a span of control of roughly three to seven direct reports, and common terminology so unfamiliar agencies can coordinate. For an NP the practical point is that you take direction from an assigned role, not your usual reporting line.

#425 · Leader

How does mass casualty triage invert everyday clinical ethics, and what does START sort on?

Answer

Normal practice gives the most resources to the sickest patient. Mass casualty triage seeks the greatest good for the greatest number, so catastrophic injuries with low survival chance may be designated expectant and given comfort measures. START sorts by respirations, perfusion and mental status into immediate, delayed, minor and expectant, with continuous reassessment.

#426 · Educator

Which assumptions of Knowles' andragogy change how a nurse practitioner teaches adults?

Answer

Adults bring a store of experience that new information must connect to. They are self directing and resist being managed. They become ready to learn when a life problem makes the learning necessary. They are problem centred rather than subject centred, and driven more by internal motivation than outside pressure.

#427 · Educator

A client has just received a cancer diagnosis. What does readiness to learn tell you to do?

Answer

Expect that little said in the next ten minutes is retained, and that repeating it louder does not help. Give one or two anchor points, write them down, and schedule a return contact. Providing written information with follow up within 48 hours often beats explaining all treatment options at that visit.

#428 · Educator

Name the four phases of Kolb's experiential learning cycle and what they imply for skill teaching.

Answer

Concrete experience, reflective observation, abstract conceptualization, active experimentation. Learning sticks when the person does something, reflects, forms a rule, and tries again, which is why having a client draw up and inject saline in front of you beats any pamphlet on injection technique.

#429 · Educator

When teaching a parent about a newborn, how much do you cover and what gets priority?

Answer

The learner is not the patient. Limit each contact to three key messages and prioritize the ones that prevent death: safe sleep positioning, never shaking an infant, and the fever thresholds requiring urgent assessment. Ask what the parent has already been told, since you may need to correct a family practice.

#430 · Educator

What consent and confidentiality rules shape teaching an adolescent about sexual health?

Answer

Canadian practice generally rests on the mature minor doctrine, where a young person who understands the nature and consequences of a treatment can consent, with rules varying by province or territory. Quebec sets 14 as the age of independent consent to care. Offer time alone, and state clearly what stays private and what does not.

#431 · Educator

Which adjustments come first when teaching an older adult with sensory or cognitive change?

Answer

Environmental adjustment before content adjustment: face the person, reduce background noise, check hearing aids, speak at a normal pace rather than shouting. Use large print of at least 14 point, high contrast, matte paper. For cognitive change, shorten sessions and repeat across visits. Slowness of hearing is not slowness of thought.

#432 · Educator

Define health literacy and state what Canadian survey data has found about its prevalence.

Answer

The ability to find, understand and use health information to make decisions. Canadian survey data has repeatedly found roughly 60 percent of adults, and about 88 percent of adults over 65, score below the level considered adequate for everyday health demands.

#433 · Educator

What indirect signals suggest a client has limited health literacy?

Answer

Limited health literacy is concealed, and hidden best from professionals. Signals include saying I forgot my glasses when handed a form, taking paperwork home to review with a daughter, or filling a prescription and never starting it. Nodding is not understanding.

#434 · Educator

What is the universal precautions approach to health literacy, and why is it recommended?

Answer

Teach everyone in a way that works for limited literacy, using the same logic as standard precautions. Because you cannot reliably identify who struggles, teaching to that level for all clients removes both the guessing and the shame.

#435 · Educator

List the plain language principles the exam expects you to recognize.

Answer

Use everyday words, saying high blood pressure rather than hypertension, and define any technical term you must use. Use short sentences and the active voice. Give numbers as natural frequencies. Lead with what the person must do. Chunk and check, delivering one piece and confirming understanding before moving on.

#436 · Educator

How should you teach a client to judge online health information?

Answer

Teach them to judge sources rather than telling them to stay offline. Useful questions: who runs the site and how is it funded, when was it updated, does it cite sources, is it selling something, and is it Canadian. Directing a client to a specific trusted Canadian page works better than a general warning.

#437 · Educator

What reading level should public written material target, and what do readability formulas miss?

Answer

Grade 6 to grade 8, and lower for populations with known literacy barriers. Formulas such as Flesch Kincaid measure only sentence and word length, so they cannot judge layout, images or cultural fit. Testing a draft with a few actual clients tells you more than any formula.

#438 · Educator

What does teach back actually test, and why does that framing matter?

Answer

It tests the clarity of the teaching, not the intelligence of the learner. The client explains in their own words what they will do; if they cannot, the explanation failed and you try a different one. Framed that way the technique carries no shame and clients accept it readily.

#439 · Educator

Contrast poor and good teach back wording.

Answer

Poor wording invites a useless yes: do you understand, any questions, you will take it twice a day right. Good wording puts responsibility on you: I want to make sure I explained this clearly, can you tell me how you are going to take this new medicine when you get home.

#440 · Educator

A client's teach back version is incomplete. What is the next step?

Answer

Do not repeat the same words louder. Re explain differently, using a drawing, a demonstration or a different comparison, then check again. That loop is the whole method, and it is where most clinicians stop too early.

#441 · Educator

Why is teach back not enough for an inhaler, injection, dressing or glucometer?

Answer

Teach back is not show back. For any device or hands on task, ask for a return demonstration, because verbal accuracy does not predict manual accuracy.

#442 · Educator

Which learning objectives can be assessed, and which cannot?

Answer

Demonstrate, state, list and choose are measurable. Know, understand and appreciate are not. The client will understand diabetes cannot be assessed; the client will demonstrate glucometer use and state two symptoms of hypoglycemia and the action for each can be.

#443 · Educator

What is the most common failure when a clinician builds an educational resource?

Answer

Building it from a clinician's assumption. Start with a needs assessment covering what learners already know, what they believe, the real barriers, and what they say they want, and involve the target group in the design.

#444 · Educator

Who may interpret for a client, and what must be documented?

Answer

Use professional interpreters, never a family member and never a child. Family interpreters filter, soften and omit, and using a child inverts the family structure. Document that an interpreter was used and how. Where in person interpretation is unavailable, telephone and video interpretation are accepted alternatives.

#445 · Educator

What does treating accessibility as a design requirement rather than an add on look like?

Answer

Large print, plain fonts, high contrast, screen reader compatible documents, captions on video, and formats usable by people with low vision, hearing loss or limited dexterity. Ontario's Accessibility for Ontarians with Disabilities Act is the best known of several provincial statutes requiring it.

#446 · Educator

Name Kirkpatrick's four levels of educational evaluation and how each is measured.

Answer

Reaction, whether learners found it relevant and satisfying, the weakest evidence. Learning, whether knowledge, skill or confidence changed, measured by teach back, return demonstration or a short pre and post quiz. Behaviour, whether people did anything differently, measured by refill records or observed technique. Results, whether client or system outcomes changed.

#447 · Educator

Attendance at your group education session is 20 percent. What does Kirkpatrick thinking tell you?

Answer

Evaluate the delivery method, not only the learner. Attendance that low points to scheduling or transport, not to motivation. Fixing the format is the intervention, and it also feeds a quality improvement cycle.

#448 · Educator

Name the five steps of the one minute preceptor model and why the order works.

Answer

Get a commitment by asking what the learner thinks is going on. Probe for supporting evidence. Teach one transferable general rule. Reinforce what was done well, specifically. Correct errors plainly. It works because the learner commits before you supply the answer.

#449 · Educator

What makes feedback to a learner usable, and what should you ask for first?

Answer

Timely, specific, behaviour focused and paired with a plan. You interrupted the client three times in the first two minutes, so we lost the story; next patient, try 60 seconds of silence after your opening question. Work on your communication is not usable. Ask the learner to self assess first, which produces less defensiveness.

#450 · Educator

A client repeats a false health claim they read online. How do you respond?

Answer

Ridicule and flat contradiction end the conversation and harden the belief. Ask what they read and where, acknowledge the underlying concern, ask permission to share what the Canadian evidence shows, correct briefly without repeating the false claim, and leave the decision with the client. Preserving the relationship keeps the second conversation possible.

#451 · Advocate

How does the CNPLE blueprint define cultural safety?

Answer

Practising in a manner that affirms, responds to and fosters the cultural expression of clients, and that addresses the power relationships between the service provider and the people who use the service. It requires reflection on one's own cultural identity.

#452 · Advocate

Why is power the centre of cultural safety rather than knowledge of other people's customs?

Answer

Cultural safety is about the imbalance of power between clinician and client and what the clinician does with it. You hold the prescription pad, the referral, the diagnostic label, and the power to decide whether a complaint counts as real. An intervention ignoring that imbalance is not culturally safe no matter how warm the tone.

#453 · Advocate

Who decides whether an encounter was culturally safe?

Answer

The client, not the provider. This point generates more exam questions than any other in the domain. If the client experienced the encounter as unsafe, it was unsafe, and your intent is not the measure.

#454 · Advocate

An NP says: I treat everyone the same, so my care is culturally safe. What two errors is that?

Answer

Self certifying, when cultural safety is judged by the client rather than claimed by the provider. And confusing sameness with equity, since giving everyone the identical service produces worse outcomes for people who need something different.

#455 · Advocate

How does the blueprint define unsafe cultural practice?

Answer

Any action that demeans, diminishes or disempowers a person's cultural identity and well being. The test sits on the effect on the person, not on whether the clinician meant harm.

#456 · Advocate

Distinguish cultural awareness from cultural sensitivity.

Answer

Cultural awareness is noticing that difference exists. It is the shallowest level and coexists with stereotyping, since noticing a group exists says nothing about how you treat its members. Cultural sensitivity adds respect for difference and the start of self reflection, but it still sits inside the clinician's attitude.

#457 · Advocate

What is wrong with framing the goal as cultural competence?

Answer

It frames culture as a body of knowledge a clinician can acquire, as though enough training makes someone proficient in a culture. Competence implies an endpoint, treats cultures as fixed and uniform, invites checklist thinking about what those clients believe, and leaves the power imbalance untouched.

#458 · Advocate

How does cultural safety differ from awareness, sensitivity and competence?

Answer

It shifts focus from the clinician's knowledge to the client's experience and to the structures around the encounter. Of the four terms it is the only one assessed by the person receiving care.

#459 · Advocate

What three features define cultural humility?

Answer

Ongoing reflection on your own cultural identity and assumptions, including assumptions about time, family decision making, eye contact and pain expression. Actively redressing power imbalance. And institutional accountability, not only personal, because a humble clinician inside a hostile system still produces harm.

#460 · Advocate

A stem says an NP finished cultural safety training three years ago and therefore practises safely. Correct?

Answer

No. Cultural humility is a lifelong process, not a credential, and there is no completion state. A described training certificate never establishes that current practice is culturally safe.

#461 · Advocate

An NP asks a client what matters, then proceeds with the original plan. What is the problem?

Answer

Asking and then proceeding unchanged is theatre. Redressing power imbalance means asking what matters and building the plan around the answer, so the client's response actually changes what happens next.

#462 · Advocate

Define implicit bias and explain why calling it racism ends the conversation.

Answer

The automatic association between a social group and a set of attributes, operating outside conscious awareness and often contradicting a person's stated values. It is not a synonym for prejudice, and treating it as an accusation of racism ends the conversation before it starts.

#463 · Advocate

Under which working conditions does implicit bias operate hardest?

Answer

Time pressure, cognitive load, fatigue and ambiguity, which are exactly the conditions that define nurse practitioner work. When you decide quickly with imperfect data, the fast associative system fills the gaps. The risk rises on your worst shift, not your best.

#464 · Advocate

Name the documented clinical effects of implicit bias, including the one affecting clients with a known label.

Answer

Pain is systematically undertreated in Black, Indigenous and other racialized clients. Women's cardiac symptoms are more often attributed to anxiety and investigated less aggressively. Clients with a history of substance use or mental illness experience diagnostic overshadowing, where new symptoms are read as an extension of the known label rather than investigated on their merits.

#465 · Advocate

Which anti bias interventions are weak, and why is colour blindness counterproductive?

Answer

Weak: one off implicit bias training as a standalone fix, taking an implicit association test and stopping there, and trying to become colour blind. Colour blindness is counterproductive because refusing to see race means refusing to see racism and the outcomes it produces.

#466 · Advocate

Which structural moves actually reduce the effect of implicit bias on clinical decisions?

Answer

Slow down at the decision point, particularly when you feel certain. Standardize the process, since checklists and defined criteria for investigation leave less room for the discretion where bias lives. Run the counterfactual: would you order the same test if this client were wealthier, white, male or housed. Audit outcome data by group.

#467 · Advocate

How does documentation language spread bias, and which words are the warning signs?

Answer

Words like non compliant, difficult and drug seeking transfer your bias to the next clinician, who reads the chart before meeting the person. Stigmatizing chart language lowers the quality of care that clinician provides.

#468 · Advocate

Is implicit bias something a clinician can eliminate?

Answer

No. Bias is managed, not cured. Build systems that catch it rather than claiming you have eliminated it, and treat any claim of having eliminated it as the wrong answer.

#469 · Advocate

Define the social determinants of health and name the ones that matter most in Canada.

Answer

The conditions in which people are born, grow, live, work and age, and the systems put in place to deal with illness. In Canada these include income, education, employment, early childhood development, food insecurity, housing, social exclusion, the social safety net, Indigenous status, gender, race and disability. They drive more variation in population health than clinical care does.

#470 · Advocate

Distinguish equality from equity, and say which the blueprint asks for.

Answer

Equality means giving everyone the same thing. Equity means giving people what they need to reach a comparable outcome, which necessarily means giving different people different amounts. Practice statement V.3 asks for equitable care, not equal care.

#471 · Advocate

What does equity look like in a clinic that currently offers everyone the same fifteen minute appointment?

Answer

Longer appointments for complex clients, interpretation at no cost, teach back instead of handouts, phone booking kept alongside the online portal, and outreach rather than discharge when appointments are missed. The identical service produces a far worse outcome for a client who reads at a grade four level and takes two buses to get there.

#472 · Advocate

What is the equity trap in exam stems, and give examples of policies that create it.

Answer

Neutral sounding policies that produce unequal effects: a three strikes no show discharge rule, a photo identification requirement, mandatory online forms, or a rule that clients attend alone. Each filters out the people who need care most.

#473 · Advocate

What causes the health inequities experienced by First Nations, Inuit and Metis Peoples?

Answer

Colonization, not culture, genetics or personal choice. A nurse practitioner who cannot state that plainly cannot practise safely in this country. Any answer option attributing the gap to culture or individual choices is wrong.

#474 · Advocate

What is the Indian Act and how does it still affect health access?

Answer

First passed in 1876 and still in force in amended form, it gave the federal government control over identity, land, governance and family life. Its status provisions still affect who can access which programs, which is why jurisdictional coverage questions turn on it.

#475 · Advocate

What health effects followed residential schools and the Sixties Scoop?

Answer

Residential schools operated for more than a century, the last federally supported school closing in 1996. They separated children from family, language and land, producing measurable intergenerational effects on health, parenting and trust in institutions. The Sixties Scoop and the overrepresentation of Indigenous children in child welfare extended the same pattern of removal.

#476 · Advocate

Who was Jordan River Anderson and why does the principle carry his name?

Answer

A young boy from Norway House Cree Nation in Manitoba who had complex medical needs and died in hospital while federal and provincial governments argued over which of them would pay for his home based care.

#477 · Advocate

State the rule Jordan's Principle sets and what an NP must do with it.

Answer

It is a child first principle: the government of first contact pays for the service the First Nations child needs and settles the jurisdictional dispute afterward. It has been affirmed and enforced through Canadian Human Rights Tribunal rulings. Do not delay a needed service while funding responsibility is sorted out. A parallel initiative covers Inuit children.

#478 · Advocate

How many Calls to Action did the Truth and Reconciliation Commission release, when, and which address health?

Answer

94 Calls to Action, released in 2015. Calls to Action 18 through 24 address health.

#479 · Advocate

In substance, what do the first of the Truth and Reconciliation Commission's health Calls to Action ask governments to do?

Answer

Acknowledge that the current state of Indigenous health is a direct result of previous Canadian government policies and implement Indigenous health care rights, then identify and close measurable health outcome gaps with annual public reporting.

#480 · Advocate

What do the remaining Truth and Reconciliation Commission health Calls to Action ask of the health professions?

Answer

Recognize the value of Indigenous healing practices and fund healing centres, increase the number of Indigenous health professionals, provide cultural competency training for all health care professionals, and require medical and nursing schools to teach a course on Indigenous health covering residential schools, Treaties and Indigenous rights, with skills based training in intercultural competency and anti racism.

#481 · Advocate

What was the In Plain Sight report and what did it find?

Answer

Released beginning in November 2020 under Dr. Mary Ellen Turpel Lafond, it investigated Indigenous specific racism in British Columbia health care. Drawing on input from roughly 9,000 people, it found widespread systemic racism, including a very high proportion of Indigenous respondents reporting discrimination that discouraged them from seeking care, and issued 24 recommendations.

#482 · Advocate

What is Joyce's Principle and what events led to it?

Answer

Named for Joyce Echaquan, an Atikamekw woman who died in a Quebec hospital in 2020 while recording staff mocking her. Put forward by the Atikamekw of Manawan and the Atikamekw Nation Council, it asserts the right of Indigenous people to equitable access to social and health services without discrimination, and to the highest attainable standard of health, including traditional medicines.

#483 · Advocate

What does the United Nations Declaration on the Rights of Indigenous Peoples Act require, and when did it pass?

Answer

It became federal law in 2021 and requires the Government of Canada to align federal laws with the Declaration and to develop an action plan. British Columbia enacted its own legislation to the same end in 2019. Free, prior and informed consent, self determination and the right to traditional medicines flow from it into Canadian health policy.

#484 · Advocate

In daily practice, what does culturally safe care for an Indigenous client actually look like?

Answer

Asking rather than assuming about identity, community and traditional practices, making room for family and Elders, treating a client's wariness as an earned response to a documented pattern, and never treating traditional medicine as an obstacle.

#485 · Advocate

Which five things are separate questions when taking an inclusive sexual and gender history?

Answer

Sexual orientation, gender identity, sex assigned at birth, current anatomy, and current partners. One answer predicts none of the others, so each must be asked rather than inferred.

#486 · Advocate

What determines which screening a trans or non binary client needs?

Answer

Screen based on the organs the client actually has, not the gender marker in the chart. The chart marker is an administrative field and says nothing about what anatomy is present.

#487 · Advocate

How should a nurse practitioner handle names and pronouns, including on requisitions?

Answer

Use the client's correct name and pronouns in speech, in the chart and in referrals, regardless of what legal documents say. Record both chosen and legal name where billing or laboratory matching requires it, so a requisition does not out the client. If you slip, correct yourself and move on, since a long apology makes the client manage your discomfort.

#488 · Advocate

What does gender affirming care include, and which model of access is the prevailing direction?

Answer

Social, medical and surgical elements, and not every client wants all three. Nurse practitioners in most Canadian jurisdictions can initiate and monitor gender affirming hormone therapy within their scope. An informed consent model rather than a gatekeeping model is the prevailing direction, because treating a client's gender as a condition requiring your approval reproduces the power imbalance.

#489 · Advocate

Name the documented health care barriers trans and non binary clients report.

Answer

Refusal of care, intrusive questions unrelated to the visit, every complaint attributed to hormones or gender itself, disclosure to staff with no clinical need to know, unsafe inpatient room assignment, and long waits for specialized services. Avoidance of care after prior mistreatment is common and is itself a health risk.

#490 · Advocate

Who does the Interim Federal Health Program cover, and what is the practical barrier to using it?

Answer

It provides limited, temporary coverage to protected persons including resettled refugees and refugee claimants not yet eligible for provincial or territorial insurance, covering basic care and, depending on category, supplemental and prescription drug coverage. Not every provider bills it, which is itself a barrier.

#491 · Advocate

What barriers do clients with disabilities face, and what two rules govern the encounter?

Answer

Barriers are physical, communicational, attitudinal and systemic. Speak to the client, not the companion. Ask what accommodation is needed rather than deciding for them.

#492 · Advocate

What practical equity measures fit clients experiencing homelessness?

Answer

Expect competing survival priorities, no fixed address for follow up, lost medications, and appointment structures that assume a stable life. Drop in access, dispensing that accounts for storage and theft, and treating a missed appointment as a system failure rather than the client's fault.

#493 · Advocate

What is the Canadian standard of care for people who use drugs?

Answer

Harm reduction. Abstinence is neither a precondition for treatment nor a realistic universal goal. Stigmatizing chart language lowers the quality of care the next clinician provides.

#494 · Advocate

What shapes care for rural and remote clients and for official language minority communities?

Answer

For rural and remote clients: distance, thin services, limited specialist access, weather and travel cost, so a plan the client cannot reach is not a plan. Francophones outside Quebec and Anglophones in Quebec have recognized rights to health services in their language, and language of service is a determinant of safety, not a courtesy.

#495 · Advocate

In a community of 700 where you treat your neighbour, what is the duty regarding dual relationships?

Answer

Avoidance is impossible, so the obligation shifts to management. Name the overlap out loud with the client, keep clinical information strictly inside the clinical setting, hold documentation to a higher standard, and decline encounters where objectivity cannot survive, such as prescribing controlled substances to a close family member. Follow your regulator's standards.

#496 · Advocate

Give examples of individual advocacy and state its limit.

Answer

Securing an interpreter, appealing a denied drug benefit, writing the letter supporting a disability application, or documenting a refusal of care so it becomes visible. Its limit is that it solves the case and leaves the cause intact.

#497 · Advocate

When a stem describes a recurring barrier rather than one incident, which level of advocacy is keyed?

Answer

Organizational advocacy, which targets the pattern. If you write the same appeal letter fifteen times, the appeal process is the problem. Bring aggregate data to a manager, propose a change to intake or booking, join a quality committee, or revise a policy that filters out the clients who need care most. This is realistic entry level work.

#498 · Advocate

What routes does policy advocacy use, and whose voice must lead any advocacy?

Answer

Professional associations, regulatory college consultations, briefs to government, coalition work with affected communities and, where employment terms permit, public comment. Individual nurse practitioners rarely change legislation alone, but organized ones do. Advocacy is done with clients, not over them, so ask what the client wants advocated for and respect a decision not to escalate.

#499 · Advocate

When does a conflict of interest exist, and what forms does it take in practice?

Answer

When a competing interest could reasonably be seen to influence your professional judgment. The standard is reasonable perception and potential influence, not proof your judgment changed. Forms include industry gifts, meals, sponsored education and speaking fees, self referral to a service you own, dual relationships, research recruitment, and institutional throughput targets.

#500 · Advocate

Why is telling a client you own the clinic you are referring them to not enough?

Answer

Disclosure alone is not management. It informs the client but leaves the conflict operating and transfers responsibility to the person with the least power in the room. Management means removing the conflict, recusing yourself, offering genuine alternatives, arranging independent oversight, or declining the benefit. Disclose as well, always, but in addition to managing rather than instead.